

SWHS 350: ETHICS IN SOCIAL WORK/ HUMAN SERVICES (BAKER-NAUMAN)



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Services (Baker-Nauman)

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1.1: Helping those in need- Human Service Workers

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1.2: Kohlberg's Stages of Moral Development

Moral Development: Forming a Sense of Rights and Responsibilities

Morality is a system of beliefs about what is right and good compared to what is wrong or bad. **Moral development** refers to changes in moral beliefs as a person grows older and gains maturity. Moral beliefs are related to, but not identical with, moral *behavior*: it is possible to know the right thing to do, but not actually do it. It is also not the same as knowledge of *social conventions*, which are arbitrary customs needed for the smooth operation of society. Social conventions may have a moral element, but they have a primarily practical purpose. Conventionally, for example, motor vehicles all keep to the same side of the street (to the right in the United States, to the left in Great Britain). The convention allows for smooth, accident-free flow of traffic. But following the convention also has a moral element, because an individual who chooses to drive on the wrong side of the street can cause injuries or even death. In this sense, choosing the wrong side of the street is wrong morally, though the choice is also unconventional.

When it comes to schooling and teaching, moral choices are not restricted to occasional dramatic incidents, but are woven into almost every aspect of classroom life. Imagine this simple example. Suppose that you are teaching, reading to a small group of second-graders, and the students are taking turns reading a story out loud. Should you give every student the same amount of time to read, even though some might benefit from having additional time? Or should you give more time to the students who need extra help, even if doing so bores classmates and deprives others of equal shares of “floor time”? Which option is more fair, and which is more considerate? Simple dilemmas like this happen every day at all grade levels simply because students are diverse, and because class time and a teacher’s energy are finite.

Embedded in this rather ordinary example are moral themes about fairness or justice, on the one hand, and about consideration or care on the other. It is important to keep both themes in mind when thinking about how students develop beliefs about right or wrong. A **morality of justice** is about human rights—or more specifically, about respect for fairness, impartiality, equality, and individuals’ independence. A **morality of care**, on the other hand, is about human responsibilities—more specifically, about caring for others, showing consideration for individuals’ needs, and interdependence among individuals. Students and teachers need both forms of morality. In the next sections therefore we explain a major example of each type of developmental theory, beginning with the morality of justice.

Kohlberg's morality of justice

One of the best-known explanations of how morality of justice develops was developed by Lawrence Kohlberg and his associates (Kohlberg, Levine, & Hower, 1983; Power, Higgins, & Kohlberg, 1991). Using a stage model similar to Piaget’s, Kohlberg proposed six stages of moral development, grouped into three levels. Individuals experience the stages universally and in sequence as they form beliefs about justice. He named the levels simply preconventional, conventional, and (you guessed it) postconventional. The levels and stages are summarized in Table 1.

Table 1: Moral stages according to Kohlberg

Moral stage	Definition of what is “good”
<i>Preconventional Level</i>	
Stage 1: Obedience and punishment	Action that is rewarded and <i>not</i> punished
Stage 2: Market exchange	Action that is agreeable to the child and child’s partner
<i>Conventional Level</i>	
Stage 3: Peer opinion	Action that wins approval from friends or peers
Stage 4: Law and order	Action that conforms to the community customs or laws
<i>Postconventional Level</i>	
Stage 5: Social contract	Action that follows socially accepted ways of making decisions
Stage 6: Universal principles	Action that is consistent with self-chosen, general principles

Preconventional justice: obedience and mutual advantage

The *preconventional* level of moral development coincides approximately with the preschool period of life and with Piaget's preoperational period of thinking. At this age the child is still relatively self-centered and insensitive to the moral effects of actions on others. The result is a somewhat short-sighted orientation to morality. Initially (Kohlberg's Stage 1), the child adopts an **ethics of obedience and punishment**—a sort of “morality of keeping out of trouble.” The rightness and wrongness of actions is determined by whether actions are rewarded or punished by authorities such as parents or teachers. If helping yourself to a cookie brings affectionate smiles from adults, then taking the cookie is considered morally “good.” If it brings scolding instead, then it is morally “bad.” The child does not think about why an action might be praised or scolded; in fact, says Kohlberg, he would be incapable at Stage 1 of considering the reasons even if adults offered them.

Eventually the child learns not only to respond to positive consequences, but also learns how to *produce* them by exchanging favors with others. The new ability creates Stage 2, an **ethics of market exchange**. At this stage the morally “good” action is one that favors not only the child, but another person directly involved. A “bad” action is one that lacks this reciprocity. If trading the sandwich from your lunch for the cookies in your friend's lunch is mutually agreeable, then the trade is morally good; otherwise it is not. This perspective introduces a type of fairness into the child's thinking for the first time. But it still ignores the larger context of actions—the effects on people not present or directly involved. In Stage 2, for example, it would also be considered morally “good” to pay a classmate to do another student's homework—or even to avoid bullying or to provide sexual favors—provided that both parties regard the arrangement as being fair.

Conventional justice: conformity to peers and society

As children move into the school years, their lives expand to include a larger number and range of peers and (eventually) of the community as a whole. The change leads to *conventional morality*, which are beliefs based on what this larger array of people agree on—hence Kohlberg's use of the term “conventional.” At first, in Stage 3, the child's reference group are immediate peers, so Stage 3 is sometimes called the **ethics of peer opinion**. If peers believe, for example, that it is morally good to behave politely with as many people as possible, then the child is likely to agree with the group and to regard politeness as not merely an arbitrary social convention, but a moral “good.” This approach to moral belief is a bit more stable than the approach in Stage 2, because the child is taking into account the reactions not just of one other person, but of many. But it can still lead astray if the group settles on beliefs that adults consider morally wrong, like “Shop lifting for candy bars is fun and desirable.”

Eventually, as the child becomes a youth and the social world expands even more, he or she acquires even larger numbers of peers and friends. He or she is therefore more likely to encounter disagreements about ethical issues and beliefs. Resolving the complexities lead to Stage 4, the **ethics of law and order**, in which the young person increasingly frames moral beliefs in terms of what the majority of society believes. Now, an action is morally good if it is legal or at least customarily approved by most people, including people whom the youth does not know personally. This attitude leads to an even more stable set of principles than in the previous stage, though it is still not immune from ethical mistakes. A community or society may agree, for example, that people of a certain race should be treated with deliberate disrespect, or that a factory owner is entitled to dump waste water into a commonly shared lake or river. To develop ethical principles that reliably avoid mistakes like these require further stages of moral development.

Postconventional justice: social contract and universal principles

As a person becomes able to think abstractly (or “formally,” in Piaget's sense), ethical beliefs shift from acceptance of what the community *does* believe to the *process* by which community beliefs are formed. The new focus constitutes Stage 5, the **ethics of social contract**. Now an action, belief, or practice is morally good if it has been created through fair, democratic processes that respect the rights of the people affected. Consider, for example, the laws in some areas that require motorcyclists to wear helmets. In what sense are the laws about this behavior ethical? Was it created by consulting with and gaining the consent of the relevant people? Were cyclists consulted and did they give consent? Or how about doctors or the cyclists' families? Reasonable, thoughtful individuals disagree about how thoroughly and fairly these *consultation* processes should be. In focusing on the processes by which the law was created, however, individuals are thinking according to Stage 5, the ethics of social contract, regardless of the position they take about wearing helmets. In this sense, beliefs on both sides of a debate about an issue can sometimes be morally sound even if they contradict each other.

Paying attention to due process certainly seems like it should help to avoid mindless conformity to conventional moral beliefs. As an ethical strategy, though, it too can sometimes fail. The problem is that an ethics of social contract places more faith in democratic process than the process sometimes deserves, and does not pay enough attention to the content of what gets decided. In

principle (and occasionally in practice), a society could decide democratically to kill off every member of a racial minority, for example, but would deciding this by due process make it ethical? The realization that ethical means can sometimes serve unethical ends leads some individuals toward Stage 6, the **ethics of self-chosen, universal principles**. At this final stage, the morally good action is based on personally held principles that apply both to the person's immediate life as well as to the larger community and society. The universal principles may include a belief in democratic due process (Stage 5 ethics), but also other principles, such as a belief in the dignity of all human life or the sacredness of the natural environment. At Stage 6, the universal principles will guide a person's beliefs even if the principles mean disagreeing occasionally with what is customary (Stage 4) or even with what is legal (Stage 5).

Gilligan's morality of care

As logical as they sound, Kohlberg's stages of moral justice are not sufficient for understanding the development of moral beliefs. To see why, suppose that you have a student who asks for an extension of the deadline for an assignment. The justice orientation of Kohlberg's theory would prompt you to consider issues of whether granting the request is fair. Would the late student be able to put more effort into the assignment than other students? Would the extension place a difficult demand on you, since you would have less time to mark the assignments? These are important considerations related to the rights of students and the teacher. In addition to these, however, are considerations having to do with the responsibilities that you and the requesting student have for each other and for others. Does the student have a valid personal reason (illness, death in the family, etc.) for the assignment being late? Will the assignment lose its educational value if the student has to turn it in prematurely? These latter questions have less to do with fairness and rights, and more to do with taking care of and responsibility for students. They require a framework different from Kohlberg's to be understood fully.

One such framework has been developed by Carol Gilligan, whose ideas center on a **morality of care**, or system of beliefs about human responsibilities, care, and consideration for others. Gilligan proposed three moral positions that represent different extents or breadth of ethical care. Unlike Kohlberg, Piaget, or Erikson, she does not claim that the positions form a strictly developmental sequence, but only that they can be ranked hierarchically according to their depth or subtlety. In this respect her theory is "semi-developmental" in a way similar to Maslow's theory of motivation (Brown & Gilligan, 1992; Taylor, Gilligan, & Sullivan, 1995). Table 2 summarizes the three moral positions from Gilligan's theory

Table 2: Positions of moral development according to Gilligan

Moral position	Definition of what is morally good
Position 1: Survival orientation	Action that considers one's personal needs only
Position 2: Conventional care	Action that considers others' needs or preferences, but not one's own
Position 3: Integrated care	Action that attempts to coordinate one's own personal needs with those of others

Position 1: caring as survival

The most basic kind of caring is a **survival orientation**, in which a person is concerned primarily with his or her own welfare. If a teenage girl with this ethical position is wondering whether to get an abortion, for example, she will be concerned entirely with the effects of the abortion on herself. The morally good choice will be whatever creates the least stress for herself and that disrupts her own life the least. Responsibilities to others (the baby, the father, or her family) play little or no part in her thinking.

As a moral position, a survival orientation is obviously not satisfactory for classrooms on a widespread scale. If every student only looked out for himself or herself, classroom life might become rather unpleasant! Nonetheless, there are situations in which focusing primarily on yourself is both a sign of good mental health and relevant to teachers. For a child who has been bullied at school or sexually abused at home, for example, it is both healthy and morally desirable to speak out about how bullying or abuse has affected the victim. Doing so means essentially looking out for the victim's own needs at the expense of others' needs, including the bully's or abuser's. Speaking out, in this case, requires a survival orientation and is healthy because the child is taking caring of herself.

Position 2: conventional caring

A more subtle moral position is **caring for others**, in which a person is concerned about others' happiness and welfare, and about reconciling or integrating others' needs where they conflict with each other. In considering an abortion, for example, the teenager at this position would think primarily about what other people prefer. Do the father, her parents, and/or her doctor want her to keep the child? The morally good choice becomes whatever will please others the best. This position is more demanding than Position 1, ethically and intellectually, because it requires coordinating several persons' needs and values. But it is often morally insufficient because it ignores one crucial person: the self.

In classrooms, students who operate from Position 2 can be very desirable in some ways; they can be eager to please, considerate, and good at fitting in and at working cooperatively with others. Because these qualities are usually welcome in a busy classroom, teachers can be tempted to reward students for developing and using them. The problem with rewarding Position 2 ethics, however, is that doing so neglects the student's development—his or her own academic and personal goals or values. Sooner or later, personal goals, values, and identity need attention and care, and educators have a responsibility for assisting students to discover and clarify them.

Position 3: integrated caring

The most developed form of moral caring in Gilligan's model is **integrated caring**, the coordination of personal needs and values with those of others. Now the morally good choice takes account of everyone *including* yourself, not everyone *except* yourself. In considering an abortion, a woman at Position 3 would think not only about the consequences for the father, the unborn child, and her family, but also about the consequences for herself. How would bearing a child affect her own needs, values, and plans? This perspective leads to moral beliefs that are more comprehensive, but ironically are also more prone to dilemmas because the widest possible range of individuals are being considered.

In classrooms, integrated caring is most likely to surface whenever teachers give students wide, sustained freedom to make choices. If students have little flexibility about their actions, there is little room for considering *anyone's* needs or values, whether their own or others'. If the teacher says simply: "Do the homework on page 50 and turn it in tomorrow morning," then the main issue becomes compliance, not moral choice. But suppose instead that she says something like this: "Over the next two months, figure out an inquiry project about the use of water resources in our town. Organize it any way you want—talk to people, read widely about it, and share it with the class in a way that all of us, including yourself, will find meaningful." An assignment like this poses moral challenges that are not only educational, but also moral, since it requires students to make value judgments. Why? For one thing, students must decide what aspect of the topic really matters to them. Such a decision is partly a matter of personal values. For another thing, students have to consider how to make the topic meaningful or important to *others* in the class. Third, because the time line for completion is relatively far in the future, students may have to weigh personal priorities (like spending time with friends or family) against educational priorities (working on the assignment a bit more on the weekend). As you might suspect, some students might have trouble making good choices when given this sort of freedom—and their teachers might therefore be cautious about giving such an assignment. But the difficulties in making choices are part of Gilligan's point: integrated caring is indeed more demanding than the caring based only on survival or on consideration of others. Not all students may be ready for it.

Character development: Integrating ethical understanding, care, and action

The theories described so far all offer frameworks for understanding how children grow into youth and adults. Those by Maslow, Kohlberg, and Gilligan are more specific than the one by Erikson in that they focus on the development of understanding about ethics. From a teacher's point of view, though, the theories are all limited in two ways. One problem is that they focus primarily on cognition—on what children *think* about ethical issues—more than on emotions and actions. The other is that they say little about how to encourage ethical development. Encouragement is part of teachers' jobs, and doing it well requires understanding not only what students know about ethics, but also how they feel about it and what ethical actions they are actually prepared to take.

Many educators have recognized these educational needs, and a number of them have therefore developed practical programs that integrate ethical understanding, care, and action. As a group the programs are often called **character education**, though individual programs have a variety of specific names (for example, *moral dilemma education*, *integrative ethical education*, *social competence education*, and many more). Details of the programs vary, but they all combine a focus on ethical knowledge with attention to ethical feelings and actions (Elkind & Sweet, 2004; Berkowitz & Bier, 2006; Narvaez, 2010). Character education programs goes well beyond just teaching students to obey ethical rules, such as "Always tell the whole truth" or "Always do what the teacher tells you to do." Such rules require very little thinking on the part of the student, and there are usually occasions in which a rule that is supposedly universal needs to be modified, "bent," or even disobeyed. (For example, if telling the whole truth

might hurt someone's feelings, it might sometimes be more considerate—and thus more ethical—to soften the truth a bit, or even to say nothing at all.)

Instead, character education is about inviting students to think about the broad questions of his or her life, such as “What kind of person should I be?” or “How should I live my life?” Thoughtful answers to such broad questions help to answer a host of more specific questions that have ethical implications, such as “Should I listen to the teacher right now, even if she is a bit boring, or just tune out?” or “Should I offer to help my friend with the homework she is struggling with, or hold back so that learns to do it herself?” Most of the time, there is not enough time to reason about questions like these deliberately or consciously. Responses have to become intuitive, automatic, and *embodied*—meaning that they have to be based in fairly immediate emotional responses (Narvaez, 2009). The goal of character education is to develop students' capacities to respond to daily ethical choices not only consciously and cognitively, but also intuitively and emotionally. To the extent that this goal is met, students can indeed live a good, ethically responsible life.

Schoolwide programs of character education

In the most comprehensive approaches to character education, an entire school commits itself to developing students' ethical character, despite the immense diversity among students (Minow, Schweder, & Markus, 2008). All members of the staff—not just teachers and administrators, but also custodians, and educational assistants—focus on developing positive relationships with students. The underlying theme that develops is one of cooperation and mutual care, not competition. Fairness, respect and honesty pervade class and school activities; discipline, for example, focuses on solving conflicts between students and between students and teachers, rather than on rewarding obedience or punishing wrong-doers. The approach requires significant reliance on democratic meetings and discussions, both in classrooms and wherever else groups work together in school.

Classroom programs of character education

Even if a teacher is teaching character education simply within her own classroom, there are many strategies available. The goal in this case is to establish the classroom as a place where everyone feels included, and where everyone treats everyone else with civility and respect. Conflicts and disagreements may still occur, but in a caring community they can be resolved without undue anger or hostility. Here are a few ways to work toward this sort of classroom:

- Use class meetings to decide on as many important matters as possible—such as the expected rules of behavior, important classroom activities, or ongoing disagreements.
- Try arranging for students to collaborate on significant projects and tasks.
- Arrange a “Buddies” program in which students of different grade levels work together on a significant task. Older students can sometimes assist younger students by reading to them, by listening to them read, or both. If an older student is having trouble with reading himself or herself, furthermore, a reading buddies program can sometimes also be helpful to the older student.
- Familiarize students with conflict resolution strategies, and practice using them when needed.
- Many areas of curriculum lend themselves to discussions about ethical issues. Obvious examples are certain novels, short stories, and historical events. But ethical issues lurk elsewhere as well. Teaching nutrition, for example, can raise issues about the humane treatment of animals that will be slaughtered for food, and about the ethical acceptability of using large amount of grains to feed animals even though many people in the world do not have enough to eat.
- Service learning projects can be very helpful in highlighting issues of social justice. Planning, working at and reflecting about a local soup kitchen, tutoring students from low-income families, performing simple repairs on homes in need: projects like these broaden knowledge of society and of the needs of its citizens.

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1.3: Ethical Standards for Human Services Professionals

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1.4: Making Ethical Decisions

Learning Objectives

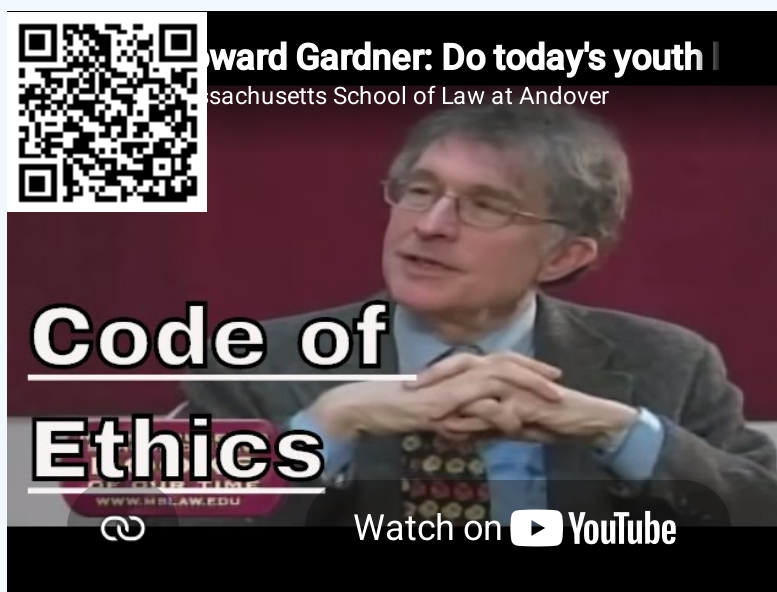
- Be able to explain the models you can use for ethical decision making.

Now that we have a working knowledge of ethics, it is important to discuss some of the models we can use to make ethical decisions. Understanding these models can assist us in developing our self-management skills and relationship management skills. These models will give you the tools to make good decisions, which will likely result in better human relations within your organization.

Note there are literally hundreds of models, but most are similar to the ones we will discuss. Most people use a combination of several models, which might be the best way to be thorough with ethical decision making. In addition, often we find ethical decisions to be quick. For example, if I am given too much change at the grocery store, I may have only a few seconds to correct the situation. In this case, our values and morals come into play to help us make this decision, since the decision making needs to happen fast.

do today's youth have a code of ethics?

Howard Gardner with University of Massachusetts Law School discusses ethics and youth



Video 1.4.1: https://www.youtube.com/watch?v=r-SE_wVoJWI.

The Twelve Questions Model

Laura Nash, an ethics researcher, created the **Twelve Questions Model** as a simple approach to ethical decision making.^[1] In her model, she suggests asking yourself questions to determine if you are making the right ethical decision. This model asks people to reframe their perspective on ethical decision making, which can be helpful in looking at ethical choices from all angles. Her model consists of the following questions:^[2]

1. Have you defined the problem accurately?
2. How would you define the problem if you stood on the other side of the fence?
3. How did this situation occur in the first place?
4. To whom and what do you give your loyalties as a person and as a member of the company?
5. What is your intention in making this decision?
6. How does this intention compare with the likely results?
7. Whom could your decision or action injure?
8. Can you engage the affected parties in a discussion of the problem before you make your decision?

9. Are you confident that your position will be as valid over a long period of time as it seems now?
10. Could you disclose without qualms your decision or action to your boss, your family, or society as a whole?
11. What is the symbolic potential of your action if understood? If misunderstood?
12. Under what conditions would you allow exceptions to your stand?

Consider the situation of Catha and her decision to take home a printer cartridge from work, despite the company policy against taking any office supplies home. She might go through the following process, using the Twelve Questions Model:

1. My problem is that I cannot afford to buy printer ink, and I have the same printer at home. Since I do some work at home, it seems fair that I can take home the printer ink.
2. If I am allowed to take this ink home, others may feel the same, and that means the company is spending a lot of money on printer ink for people's home use.
3. It has occurred due to the fact I have so much work that I need to take some of it home, and often I need to print at home.
4. I am loyal to the company.
5. My intention is to use the ink for work purposes only.
6. If I take home this ink, my intention may show I am disloyal to the company and do not respect company policies.
7. The decision could injure my company and myself, in that if I get caught, I may get in trouble. This could result in a loss of respect for me at work.
8. Yes, I could engage my boss and ask her to make an exception to the company policy, since I am doing so much work at home.
9. No, I am not confident of this. For example, if I am promoted at work, I may have to enforce this rule at some point. It would be difficult to enforce if I personally have broken the rule before.
10. I would not feel comfortable doing it and letting my company and boss know after the fact.
11. The symbolic action could be questionable loyalty to the company and respect of company policies.
12. An exception might be ok if I ask permission first. If I am not given permission, I can work with my supervisor to find a way to get my work done without having a printer cartridge at home.

As you can see from the process, Catha came to her own conclusion by answering the questions involved in this model. The purpose of the model is to think through the situation from all sides to make sure the right decision is being made.

As you can see in this model, first an analysis of the problem itself is important. Determining your true intention when making this decision is an important factor in making ethical decisions. In other words, what do you hope to accomplish and who can it hurt or harm? The ability to talk with affected parties upfront is telling. If you were unwilling to talk with the affected parties, there is a chance (because you want it kept secret) that it could be the wrong ethical decision. Also, looking at your actions from other people's perspectives is the core of this model.



Figure 1.4.1

Some of the possible approaches to ethical decision making. No one model is perfect, so understanding all of the possibilities and combining them is the best way to look at ethical decision making.

Josephson Institute of Ethics' Model

Josephson Institute of Ethics uses a model that focuses on six steps to ethical decision making. The steps consist of stop and think, clarify goals, determine facts, develop options, consider consequences, choose, and monitor/modify.

As mentioned, the first step is to stop and think. When we stop to think, this avoids rash decisions and allows us to focus on the right decision-making process. It also allows us to determine if the situation we are facing is legal or ethical. When we clarify our goals, we allow ourselves to focus on expected and desired outcomes. Next, we need to determine the facts in the situation. Where are we getting our facts? Is the person who is providing the facts to us credible? Is there bias in the facts or assumptions that may not be correct? Next, create a list of options. This can be a brainstormed list with all possible solutions. In the next step, we can look at the possible consequences of our actions. For example, who will be helped and who might be hurt? Since all ethical decisions we make may not always be perfect, considering how you feel and the outcome of your decisions will help you to make better ethical decisions in the future. Figure 1.4.2 gives an example of the ethical decision-making process using Josephson's model.

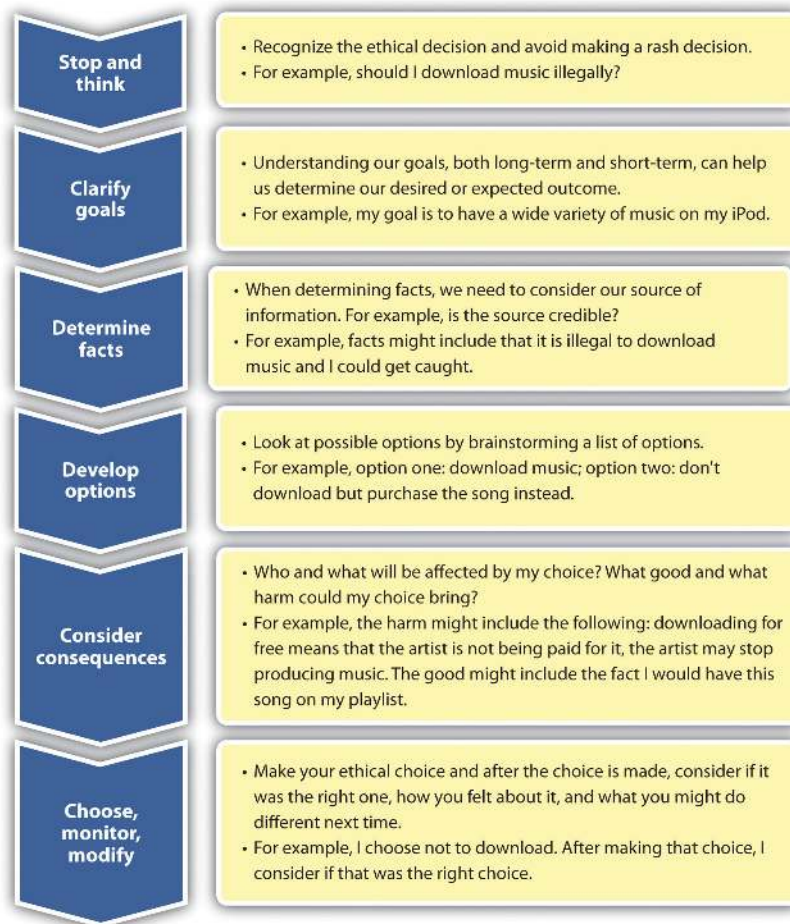


Figure 1.4.2: An Example of Josephson's Model when Dealing with the Ethical Situation of Downloading Music from Share Websites.

Steps to Ethical Decision Making

There are many models that provide several steps to the decision-making process. One such model was created in the late 1990s for the counseling profession but can apply to nearly every profession from health care to business.^[3] In this model, the authors propose eight steps to the decision-making process. As you will note, the process is similar to Josephson's model, with a few variations:

1. Step 1: Identify the problem. Sometimes just realizing a particular situation is ethical can be the important first step.

Occasionally in our organizations, we may feel that it's just the "way of doing business" and not think to question the ethical nature.

2. **Step 2: Identify the potential issues involved.** Who could get hurt? What are the issues that could negatively impact people and/or the company? What is the worst-case scenario if we choose to do nothing?
3. **Step 3: Review relevant ethical guidelines.** Does the organization have policies and procedures in place to handle this situation? For example, if a client gives you a gift, there may be a rule in place as to whether you can accept gifts and if so, the value limit of the gift you can accept.
4. **Step 4: Know relevant laws and regulations.** If the company doesn't necessarily have a rule against it, could it be looked at as illegal?
5. **Step 5: Obtain consultation.** Seek support from supervisors, coworkers, friends, and family, and especially seek advice from people who you feel are moral and ethical.
6. **Step 6: Consider possible and probable courses of action.** What are all of the possible solutions for solving the problem? Brainstorm a list of solutions—all solutions are options during this phase.
7. **Step 7: List the consequences of the probable courses of action.** What are both the positive and negative benefits of each proposed solution? Who can the decision affect?
8. **Step 8: Decide on what appears to be the best course of action.** With the facts we have and the analysis done, choosing the best course of action is the final step. There may not always be a "perfect" solution, but the best solution is the one that seems to create the most good and the least harm.

Most organizations provide such a framework for decision making. By providing this type of framework, an employee can logically determine the best course of action. The Department of Defense uses a similar framework when making decisions, as shown below.

department of defense decision-making framework

The Department of Defense uses a specific framework to make ethical decisions.^[4]

1. Define the problem.
 - a. State the problem in general terms.
 - b. State the decisions to be made
2. Identify the goals.
 - a. State short-term goals.
 - b. State long-term goals.
3. List appropriate laws or regulations.
4. List the ethical values at stake.
5. Name all the stakeholders.
 - a. Identify persons who are likely to be affected by a decision.
 - b. List what is at stake for each stakeholder.
6. Gather additional information.
 - a. Take time to gather all necessary information.
 - b. Ask questions.
 - c. Demand proof when appropriate.
 - d. Check your assumptions.
7. State all feasible solutions.
 - a. List solutions that have already surfaced.
 - b. Produce additional solutions by brainstorming with associates.
 - c. Note how stakeholders can be affected (loss or gain) by each solution.
8. Eliminate unethical options.
 - a. Eliminate solutions that are clearly unethical.
 - b. Eliminate solutions with short-term advantages but long-term problems.
9. Rank the remaining options according to how close they bring you to your goal, and solve the problem.
10. Commit to and implement the best ethical solution.

Philosopher's Approach

Philosophers and ethicists believe in a few ethical standards, which can guide ethical decision making. First, the **utilitarian approach** says that when choosing one ethical action over another, we should select the one that does the most good and least harm. For example, if the cashier at the grocery store gives me too much change, I may ask myself, if I keep the change, what harm is caused? If I keep it, is any good created? Perhaps the good created is that I am not able to pay back my friend whom I owe money to, but the harm would be that the cashier could lose his job. In other words, the utilitarian approach recognizes that some good and some harm can come out of every situation and looks at balancing the two.

In the **rights approach**, we look at how our actions will affect the rights of those around us. So rather than looking at good versus harm as in the utilitarian approach, we are looking at individuals and their rights to make our decision. For example, if I am given too much change at the grocery store, I might consider the rights of the corporation, the rights of the cashier to be paid for something I purchased, and the right of me personally to keep the change because it was their mistake.

The **common good approach** says that when making ethical decisions, we should try to benefit the community as a whole. For example, if we accepted the extra change in our last example but donated to a local park cleanup, this might be considered OK because we are focused on the good of the community, as opposed to the rights of just one or two people.

The **virtue approach** asks the question, "What kind of person will I be if I choose this action?" In other words, the virtue approach to ethics looks at desirable qualities and says we should act to obtain our highest potential. In our grocery store example, if given too much change, someone might think, "If I take this extra change, this might make me a dishonest person—which I don't want to be."

The imperfections in these approaches are threefold:^[5]

- Not everyone will necessarily agree on what is harm versus good.
- Not everyone agrees on the same set of human rights.
- We may not agree on what a common good means.

Because of these imperfections, it is recommended to combine several approaches discussed in this section when making ethical decisions. If we consider all approaches and ways to make ethical decisions, it is more likely we will make better ethical decisions. By making better ethical decisions, we improve our ability to self-manage, which at work can improve our relationships with others.

Key Takeaways

- We can use a variety of models and frameworks to help us with ethical decision making. For example, one such model is the Twelve Questions Model. This model encourages us to ask questions such as who this decision affects to determine the best ethical choice.
- Josephson's model consists of six steps. They include stop and think, clarify goals, determine facts, develop options, consider consequences, choose, and monitor/modify.
- Another model discussed has the following steps: identify the problem, identify the potential issues involved, review relevant ethical guidelines, know relevant laws and regulations, obtain consultation, consider possible and probable courses of action, list the consequences of the probable courses of action, and decide on what appears to be the best course of action.
- Philosophers look at ethical frameworks following a *utilitarian approach*, *common good approach*, *rights approach*, and the *virtue approach*. These approaches provide a framework for sound ethical decision making.

Exercises 1.4.1

1. Think of a recent ethical decision you have made. Using the model or framework of your choice, discuss how you went through the process of making a sound ethical decision.
2. What are the strengths and weaknesses of each model presented in this section? How can you combine them all to make ethical decisions?

1. Nash, L. (1981). Ethics without the sermon. *Howard Business Review*, 59 79–90, accessed February 24, 2012, www.cs.bgsu.edu/maner/heuristics/1981Nash.htm
2. Nash, L. (1981). Ethics without the sermon. *Howard Business Review*, 59 79–90, accessed February 24, 2012, www.cs.bgsu.edu/maner/heuristics/1981Nash.htm

3. Corey, G., Corey, M. S., & Callanan, P. (1998). *Issues and ethics in the helping professions*. Toronto: Brooks/Cole Publishing Company; Syracuse School of Education. (n.d.). An ethical decision making model, accessed February 24, 2012, soe.syr.edu/academic/counseli...ues/ethical_decision_making_model.aspx
4. United States Department of Defense. (1999). Joint Ethics Regulation DoD 5500.7-R., accessed February 24, 2012, csweb.cs.bgsu.edu/maner/heuri...tOfDefense.htm and ogc.hqda.pentagon.mil/EandF/Documentation/ethics_material.aspx
5. Santa Clara University. (n.d.). A framework for thinking ethically, accessed February 24, 2012, www.scu.edu/ethics/practicing...framework.html

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CHAPTER OVERVIEW

2: Chapter 2

2.1: Building a Trauma-Informed Workforce

2.2: Ethics of Practice

2.3: Stress and Coping

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2.1: Building a Trauma-Informed Workforce

IN THIS CHAPTER

- Introduction
- Workforce Recruitment, Hiring, and Retention
- Training in TIC
- Trauma-Informed Counselor Competencies
- Counselor Responsibilities and Ethics
- Clinical Supervision and Consultation
- Secondary Traumatization
- Counselor Self-Care

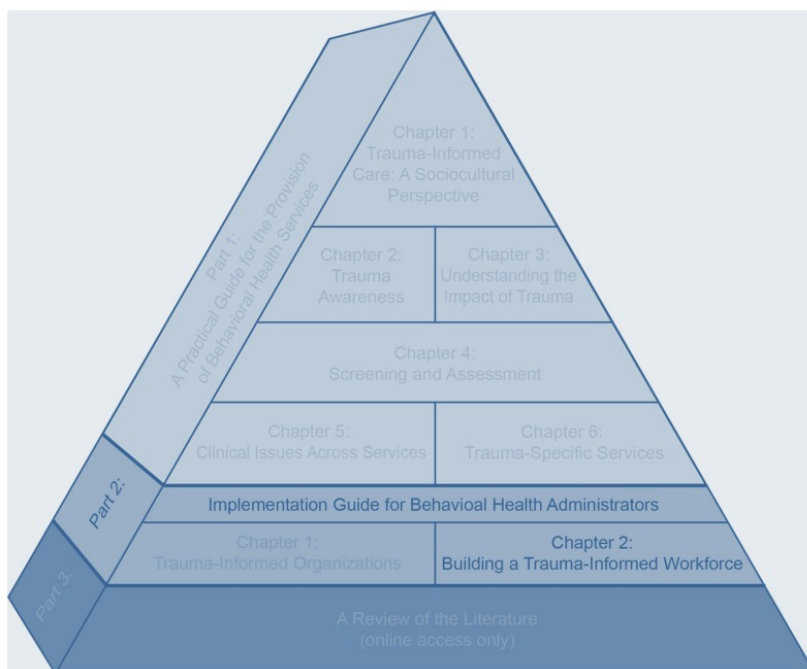
Introduction

For an organization to embrace a trauma-informed care (TIC) model fully, it must adopt a trauma-informed organizational mission and commit resources to support it. This entails implementing an agency-wide strategy for workforce development that is in alignment with the values and principles of TIC and the organization's mission statement. Without a fully trained staff, an organization will not be able to implement the TIC model. However, simply training behavioral health professionals in TIC is not enough. Counselors will not be able to sustain the kind of focus required to adopt and implement a trauma-informed philosophy and services without the ongoing support of administrators and clinical supervisors.

An organizational environment of care for the health, well-being, and safety of, as well as respect for, its staff will enhance the ability of counselors to provide the best possible trauma-informed behavioral health services to clients. This culture of care must permeate the organization from top to bottom. Behavioral health program administrators should aim to strengthen their workforce; doing so “requires creating environments that support the health and well-being, not only of persons with mental and substance use conditions, but of the workforce as well” (Hoge, 2007, p. 58). An organizational culture of care, safety, and respect demands activities that foster the development of trauma-informed counselors. This chapter focuses on key workforce development activities, such as:

- Recruiting, hiring, and retaining trauma-informed staff.
- Training behavioral health service providers on the principles of, and evidence-based and emerging best practices relevant to, TIC.
- Developing and promoting a set of counselor competencies specific to TIC.
- Delineating the responsibilities of counselors and addressing ethical considerations specifically relevant to promoting TIC.
- Providing trauma-informed clinical supervision.
- Committing to prevention and treatment of secondary trauma of behavioral health professionals within the organization.

Addressing each of these areas is essential to building a trauma-informed workforce and an organizational culture that supports TIC.



TIC Framework in Behavioral Health Services—Building a Trauma-Informed Workforce

Workforce Recruitment, Hiring, and Retention

An Action Plan for Behavioral Health Workforce Development (Hoge et al., 2007) emphasizes the importance of organization-wide support and active involvement in workforce recruitment, hiring, and retention in behavioral health systems. One of the key findings of this report is that the work environment itself in many behavioral health settings can be toxic to the workforce and may hinder the delivery of individualized, respectful, collaborative, and client-centered care to service recipients. Factors such as the downward pressure on organizations for higher productivity of counselors increase caseloads and decrease wages of behavioral health staff members and may create a high-stress environment that contributes to low morale and worker dissatisfaction. Other factors that often contribute to low retention of qualified counselors in behavioral health settings include the lack of professional career ladders, fragile job security, the lack of clinical supervision, and an inability to influence the organization in which they are working (Hoge et al., 2007).

Added to this mix is the intensity of working with people with the co-occurring conditions of trauma-related mental and substance use disorders and the risk of secondary traumatization of counselors. In creating and sustaining a trauma-informed workforce, organizations need to foster a work environment that parallels the treatment philosophy of a trauma-informed system of care. Doing so allows counselors to count on a work environment that values safety, endorses collaboration in the making of decisions at all levels, and promotes counselor well-being.

Recruitment and Hiring in a Trauma-Informed System of Care

In a 2007 technical report (Jennings, 2007b), the National Center for Trauma-Informed Care identified several priorities for organizations with regard to recruitment and hiring trauma-informed staff, including:

- Active recruitment of and outreach to prospective employees who are trauma-informed or have formal education in providing trauma-informed or trauma-specific services in settings such as universities, professional organizations, professional training and conference sites, peer support groups, and consumer advocacy groups.
- Hiring counselors and peer support staff members with educational backgrounds and training in trauma-informed and/or trauma-specific services and/or lived experience of trauma and recovery.
- Providing incentives, bonuses, and promotions for staff members during recruitment and hiring that take into consideration prospective employees' trauma-related education, training, and job responsibilities.

In addition to hiring behavioral health professionals with formal professional education and training, organizations should also “routinely survey the demographics and other characteristics of the population served and recruit a workforce of similar composition” (Hoge et al., 2007, p. 297). Essentially, this means actively engaging in outreach to consumer advocacy groups, recovery-oriented programs, community and faith-based organizations, and former clients/consumers with the intention of recruiting potential employees whose knowledge and expertise comes from their lived experience of trauma, resilience, and recovery. Support staff members, peer support workers, counselors in training, and apprentices can be recruited from this population and offered incentives, such as tuition reimbursement, training stipends, and professional mentoring with the goal of developing a trauma-informed workforce from within the demographic served. Jennings (2007b) calls these staff members “trauma champions” who can provide needed expertise in a trauma-informed organization to promote trauma-informed policies, staff development, and trauma-based services consistent with the mission of the organization (p. 135).

Who Is a Trauma Champion?

“A champion understands the impact of violence and victimization on the lives of people seeking mental health or addiction services and is a front-line worker who thinks ‘trauma first.’ When trying to understand a person’s behavior, the champion will ask, ‘is this related to abuse and violence?’ A champion will also think about whether his or her own behavior is hurtful or insensitive to the needs of a trauma survivor. The champion is there to do an identified job—he is a case manager or a counselor or a residential specialist—but in addition to his or her job, a champion is there to shine the spotlight on trauma issues.”

Source: Harris & Fallot, 2001a, p. 8.

As with hiring behavioral health professionals who are in recovery from substance use disorders, the organization should be transparent and explicit in its recruitment and hiring practices of trauma survivors in recovery. The organization can be transparent by advertising the mission statement of the organization as part of the recruitment process and inviting applicants who are in recovery from trauma to apply. The needs of behavioral health staff members who are in recovery from both substance use and trauma-related conditions and working in a trauma-informed system of care should be addressed in the organization’s ongoing training, clinical supervision, and staff development policies and practices.

Workforce Retention

Staff turnover is rampant in behavioral health settings. It is costly to the organization, and as a result, it is costly to clients. A strong therapeutic relationship with a counselor is one of the largest factors in an individual’s ability to recover from the overwhelming effects of trauma. When behavioral health professionals leave an organization prematurely or in crisis as a result of chronic levels of high stress or secondary traumatization, clients must deal with disruptions in their relationships with counselors. Some of the organizational factors that contribute to chronic levels of high stress and often lead to high staff turnover include expecting counselors to maintain high caseloads of clients who have experienced trauma; not providing trauma-informed clinical supervision and training to counselors; and failing to provide adequate vacation, health insurance, and other reasonable benefits that support counselors’ well-being. Other factors that may have a more profound impact on staff retention include failing to acknowledge the reality of secondary traumatization, promoting the view that counselors’ stress reactions are a personal failure instead of a normal response to engaging with clients’ traumatic material, and not supporting personal psychotherapy for counselors (Saakvitne, Pearlman, & Traumatic Stress Institute/Center for Adult & Adolescent Psychotherapy, 1996).

Research on promoting counselor retention in behavioral health settings demonstrates that behavioral health staff members are interested in the same kind of work environment and benefits as employees in many other fields. They include a “living wage with healthcare benefits; opportunities to grow and

advance; clarity in a job role; some autonomy and input into decisions; manageable workloads; administrative support without crushing administrative burden; basic orientation and training for assigned responsibilities; a decent and safe physical work environment; a competent and cohesive team of coworkers; the support of a supervisor; and rewards for exceptional performance” (Hoge et al., 2007, p. 18).

Advice to Administrators: Preventing Turnover and Increasing Workforce Retention

To prevent behavioral health staff turnover and increase retention of qualified, satisfied, and highly committed trauma-informed counselors, consider:

- Offering competitive wages, benefits, and performance incentives that take into account education, training, and levels of responsibility in providing trauma-informed or trauma-specific services.
- Creating a safe working environment that includes both the physical plant and policies and procedures to prevent harassment, stalking, and/or violence in the workplace and to promote respectful interactions amongst staff at all levels of the organization.
- Establishing an organizational policy that normalizes secondary trauma as an accepted part of working in behavioral health settings and views the problem as systemic—not the result of individual pathology or a deficit on the part of the counselor.
- Instituting reasonable, manageable caseloads that mix clients with and without trauma-related concerns.
- Letting staff offer input into clinical and administrative policies that directly affect their work experience.
- Providing vacation, health insurance (which includes coverage for psychotherapy/personal counseling), and other benefits that promote the well-being of the staff.
- Implementing regular, consistent clinical supervision for all clinical staff members.
- Providing ongoing training in trauma-informed services offered by the organization.

When an organization’s administration values its staff by providing competitive salaries and benefits, a safe working environment, a reasonable and manageable workload, input into the making of clinical and administrative policy decisions, and performance incentives, it helps behavioral health workers feel connected to the mission of the organization and become dedicated to its sustainability and growth. This type of work environment demonstrates both a level of respect for counselors (similar to the level of respect a trauma-informed organization displays toward clients) and an appreciation for the complexity of their job responsibilities and the stress they face when working with people who have experienced trauma in their lives. To retain behavioral health professionals working in a trauma-informed setting, wages and performance incentives should be tied not only to education, training, and work experience, but also to levels of responsibility in working with clients who have experienced trauma.

Training in TIC

Training for all staff members is essential in creating a trauma-informed organization. It may seem that training should simply focus on new counselors or on enhancing the skill level of those who have no prior experience in working with trauma, but training should, in fact, be more systematic across the organization to develop fully sustainable trauma-informed services. All employees, including administrative staff members, should receive an orientation and basic education about the prevalence of trauma and its impact on the organization’s clients. To ensure safety and reduction of harm, training should cover dynamics of retraumatization and how practice can mimic original sexual and physical abuse experiences, trigger trauma responses, and cause further harm to the person. Training for all employees must also educate them “about the impacts of culture, race, ethnicity, gender, age, sexual orientation, disability, and socioeconomic status on individuals’ experiences of trauma” (Jennings, 2007 a, p. 5).

All clinical and direct service staff members, regardless of level of experience, should receive more in depth training in screening and assessment of substance use and trauma-related disorders; the relationships among trauma, substance use disorders, and mental disorders; how to understand difficult client behaviors through a trauma-informed lens; how to avoid retraumatizing clients in a clinical setting; the development of personal and professional boundaries unique to clinical work with traumatized clients; how to identify the signs of secondary traumatization in themselves; and how to develop a comprehensive personal and professional self-care plan to prevent and/or ameliorate the effects of secondary traumatization in the workplace. All clinical staff members who work with traumatized clients should receive additional training in evidence-based and promising practices for the treatment of trauma (for information on locating training, see Appendix B.) This might include training done within the agency by experts in the field or training received by attending advanced trauma trainings. Administrators should provide the time and financial resources to clinical staff members for this professional development activity. Jennings (2007a) suggests that, whenever possible, “trainings should be multi-system, inclusive of staff in mental health and substance abuse, health care, educational, criminal justice, social services systems and agencies, and promoting systems integration and coordination” (p. 5).

Moreover, criminal justice settings, schools, military/veteran programs, and other places in which behavioral health services are provided may benefit from approaches that are sensitive to the special circumstances and cultures of these environments. For example, in exploring trauma-informed correctional care, Miller and Najavits (2012, p. 1) observe:

Prisons are challenging settings for trauma-informed care. Prisons are designed to house perpetrators, not victims. Inmates arrive shackled and are crammed into overcrowded housing units; lights are on all night, loud speakers blare without warning and privacy is severely limited. Security staff is focused on maintaining order and must assume each inmate is potentially violent. The correctional environment is full of unavoidable triggers, such as pat downs and strip searches, frequent discipline from authority figures, and restricted movement....This is likely to increase trauma-related behaviors and symptoms that can be difficult for prison staff to manage....Yet, if trauma-informed principles are introduced, all staff can play a major role in minimizing triggers, stabilizing offenders, reducing critical incidents, deescalating situations, and avoiding restraint, seclusion or other measures that may repeat aspects of past abuse.

The Need for Training

Behavioral health service providers working with clients who have mental, substance use, and trauma-related disorders need to have the best knowledge, skills, and abilities. Substance abuse counselors, in particular, require additional training and skill development to be able to extend trauma-informed

services (within the limits of their professional licensure and scope of practice) to clients who have co-occurring substance use, trauma-related, or mental disorders. Many clinical practice issues in traditional substance abuse treatment are inconsistent with trauma-informed practice, which needs to be addressed with further training. Similarly, mental health clinicians often need training in substance abuse treatment, as they typically do not have backgrounds or experience in that domain. Moreover, several surveys indicate that clinicians consistently perceive the combination of trauma and substance abuse as harder to treat than either one alone (Najavits, Norman, Kivlahan, & Kosten, 2010). It is thus key to emphasize cross-training as part of TIC. [Exhibit 2.2-1](#) addresses these issues and offers suggestions for additional training.

Case Illustration: Larry

Larry is a 28-year-old clinical social worker who just finished his master's program in social work and is working in a trauma-informed out-patient program for people with substance use disorders. He is recovering from alcohol use disorder and previously worked in a residential rehabilitation program as a recovery support counselor. There, his primary responsibilities were to take residents to Alcoholics Anonymous (AA) meetings, monitor their participation, and confront them about their substance use issues and noncompliance with the program's requirement of attendance at 12-Step meetings.

In Larry's new position as a counselor, he confronts a client in his group regarding her discomfort with attending AA meetings. The client reports that she feels uncomfortable with the idea that she has to admit that she is powerless over alcohol to be accepted by the group of mostly men. She was sexually abused by her stepfather when she was a child and began drinking heavily and smoking pot when she was 11 years old. The client reacts angrily to Larry's intervention.

In supervision, Larry discusses his concerns regarding the client's resistance to AA and the feedback that he provided to her in group. Beyond focusing supervision on Larry's new role as a counselor in a trauma-informed program, the clinical supervisor recommends that Larry take an interactive, multisession, computer-assisted training on the 12-Step facilitation (TSF) model. The TSF model introduces clients to and assists them with engaging in 12-Step recovery support groups. The agency has the computer-based training available in the office, and Larry agrees to use follow-up coaching sessions with his supervisor to work on implementation of the approach. The supervisor recognizes that Larry is falling back on his own recovery experience and the strategies he relied on in his previous counseling role. He will benefit from further training and coaching in an evidence-based practice that provides a non-aggressive, focused, and structured way to facilitate participation in recovery support groups with clients who have trauma histories.

In addition to the training needs of substance abuse counselors, all direct care workers in mental health settings, community-based programs, crisis intervention settings, and criminal justice environments should receive training in TIC. Guidelines for training in assisting trauma-exposed populations are presented in [Exhibit 2.2-2](#).

Exhibit 2.2-2

Guidelines for Training in Mental Health Interventions for Trauma-Exposed Populations

After a year of collaboration in 2002, the Task Force on International Trauma Training of the International Society for Traumatic Stress Studies published a consensus-based set of recommendations for training. Core curricular elements of the recommended training include:

- Competence in listening.
- Recognition of psychosocial and mental problems to promote appropriate assessment.
- Familiarity with established interventions in the client population.
- Full understanding of the local context, including help-seeking expectations, duration of treatment, attitudes toward intervention, cost-effectiveness of intervention, and family attitudes and involvement.
- Strategies for solving problems on the individual, family, and community levels.
- Treatment approaches for medically unexplained somatic pain.
- Collaboration with existing local resources and change agents (e.g., clergy, traditional healers, informal leaders).
- Self-care components.

Source: [Weine et al., 2002](#).

Continuing Education

Research on the effectiveness of single-session didactic and/or skill-building workshops demonstrates that immediate gains in counselor knowledge and skills diminish quickly after the training event ([Martino, Canning-Ball, Carroll, & Rounsaville, 2011](#)). Consequently, organizations may be spending their scarce financial resources on sending counselors to this kind of training but may not be reaping adequate returns with regard to long-lasting changes in counselor skills and the development of trauma-informed and trauma-specific counselor competencies. [Hoge et al. \(2007\)](#) suggest the implementation of training strategies for behavioral health professionals that have proven to be effective in improving counselor skills, attitudes, and practice approaches. These strategies include: "interactive approaches; sequenced, longitudinal learning experiences; outreach visits, known as academic detailing; auditing of practice with feedback to the learner; reminders; the use of opinion leaders to influence practice; and patient-mediated interventions, such as providing information on treatment options to persons in recovery, which in turn influences the practice patterns of their providers" (p. 124).

Advice to Administrators: Trauma-Informed Staff Training

- Establish training standards for the evidence-based and promising trauma-informed practice models (such as Seeking Safety) adopted by your organization.
- Bring expert trainers with well-developed curricula in TIC and trauma-specific practices into your organization.
- Select a core group of clinical supervisors and senior counselors to attend multisession training or certification programs. These clinicians can then train the rest of the staff.
- Use sequenced, longitudinal training experiences instead of single-session seminars or workshops.

- Emphasize interactive and experiential learning activities over purely didactic training.
- Provide ongoing mentoring/coaching to behavioral health professionals in addition to regular clinical supervision to enhance compliance with the principles and practices of TIC and to foster counselor mastery of trauma-specific practice models.
- Build organization-wide support for the ongoing integration of new attitudes and counselor skills to sustain constructive, TIC-consistent changes in practice patterns.
- Provide adequate and ongoing training for clinical supervisors in the theory and practice of clinical supervision and the principles and practices of TIC.
- Include information and interactive exercises on how counselors can identify, prevent, and ameliorate secondary traumatic stress (STS) reactions in staff trainings.
- Offer cross-training opportunities to enhance knowledge of trauma-informed processes throughout the system.

Trauma-Informed Counselor Competencies

Hoge et al. (2007) identified a number of counselor competencies in behavioral health practices that are consistent with the skills needed to be effective in a trauma-informed system of care. They include person-centered planning, culturally competent care, development of therapeutic alliances, shared responsibility for decisions, collaboratively developed recovery plans, evidence-based practices, recovery- and resilience-oriented care, interdisciplinary- and team-based practice, and consumer/client advocacy. In addition, counselor competencies critical to the effective delivery of services to clients with trauma-related disorders include:

- Screening for and assessment of trauma history and trauma-related disorders, such as mood and anxiety disorders.
- Awareness of differences between trauma-informed and trauma-specific services.
- Understanding the bidirectional relationships among substance use and mental disorders and trauma.
- Engagement in person-centered counseling.
- Competence in delivering trauma-informed and trauma-specific evidence-based interventions that lessen the symptoms associated with trauma and improve quality of life for clients.
- Awareness of and commitment to counselor self-care practices that prevent or lessen the impact of secondary traumatization on behavioral health workers.

Exhibit 2.2-3 provides a checklist of competencies for counselors working in trauma-informed behavioral health settings. Administrators and clinical supervisors can use this checklist to assess behavioral health professionals' understanding of trauma awareness and counseling skills and determine the need for additional training and clinical supervision.

Exhibit 2.2-3 Trauma-Informed Counselor Competencies Checklist

Trauma Awareness

- ___ Understands the difference between trauma-informed and trauma-specific services
- ___ Understands the differences among various kinds of abuse and trauma, including: physical, emotional, and sexual abuse; domestic violence; experiences of war for both combat veterans and survivors of war; natural disasters; and community violence
- ___ Understands the different effects that various kinds of trauma have on human development and the development of psychological and substance use issues
- ___ Understands how protective factors, such as strong emotional connections to safe and non-judgmental people and individual resilience, can prevent and ameliorate the negative impact trauma has on both human development and the development of psychological and substance use issues
- ___ Understands the importance of ensuring the physical and emotional safety of clients
- ___ Understands the importance of not engaging in behaviors, such as confrontation of substance use or other seemingly unhealthy client behaviors, that might activate trauma symptoms or acute stress reactions
- ___ Demonstrates knowledge of how trauma affects diverse people throughout their lifespans and with different mental health problems, cognitive and physical disabilities, and substance use issues
- ___ Demonstrates knowledge of the impact of trauma on diverse cultures with regard to the meanings various cultures attach to trauma and the attitudes they have regarding behavioral health treatment
- ___ Demonstrates knowledge of the variety of ways clients express stress reactions both behaviorally (e.g., avoidance, aggression, passivity) and psychologically/emotionally (e.g., hyperarousal, avoidance, intrusive memories)

Counseling Skills

- ___ Expedites client-directed choice and demonstrates a willingness to work within a mutually empowering (as opposed to a hierarchical) power structure in the therapeutic relationship
- ___ Maintains clarity of roles and boundaries in the therapeutic relationship
- ___ Demonstrates competence in screening and assessment of trauma history (within the bounds of his or her licensing and scope of practice), including knowledge of and practice with specific screening tools
- ___ Shows competence in screening and assessment of substance use disorders (within the bounds of his or her licensing and scope of practice), including knowledge of and practice with specific screening tools
- ___ Demonstrates an ability to identify clients' strengths, coping resources, and resilience
- ___ Facilitates collaborative treatment and recovery planning with an emphasis on personal choice and a focus on clients' goals and knowledge of what has previously worked for them

- ___ Respects clients' ways of managing stress reactions while supporting and facilitating taking risks to acquire different coping skills that are consistent with clients' values and preferred identity and way of being in the world
- ___ Demonstrates knowledge and skill in general trauma-informed counseling strategies, including, but not limited to, grounding techniques that manage dissociative experiences, cognitive– behavioral tools that focus on both anxiety reduction and distress tolerance, and stress management and relaxation tools that reduce hyperarousal
- ___ Identifies signs of STS reactions and takes steps to engage in appropriate self-care activities that lessen the impact of these reactions on clinical work with clients
- ___ Recognizes when the needs of clients are beyond his or her scope of practice and/or when clients' trauma material activates persistent secondary trauma or countertransference reactions that cannot be resolved in clinical supervision; makes appropriate referrals to other behavioral health professionals

Source: [Abrahams et al., 2010](#).

Counselor Responsibilities and Ethics

Treating all clients in an ethical manner is an expectation of all healthcare providers. It is of special importance when working with clients who have trauma-related disorders, as their trust in others may have been severely shaken. Counselors who work with traumatized individuals on a regular basis have special responsibilities to their clients because of the nature of this work. Administrators and clinical supervisors in trauma-informed organizations should develop policies that clearly define the counselors' job and should provide education about the role of counselors in the organization and their responsibilities to clients.

General Principles Regarding Counselor Responsibilities

The following are some general principles governing the responsibilities of counselors who provide behavioral health services for clients with histories of trauma:

- Counselors are responsible for routinely screening clients for traumatic experiences and trauma-related symptoms ([Ouimette & Brown, 2003](#); see also Treatment Improvement Protocol [TIP] 42, *Substance Abuse Treatment for Persons With Co-Occurring Disorders*, [Center for Substance Abuse Treatment \[CSAT\], 2005c](#)).
- Counselors should offer clients with substance use and trauma-related disorders continuing mental health services if it is within their professional license and scope of practice to do so.
- Counselors are responsible for referring clients with substance use disorders and co-occurring trauma-related disorders to treatment that addresses both disorders when the treatment falls outside of the counselor's professional license and scope of practice ([Ouimette & Brown, 2003](#)).
- Counselors should refer clients with substance use disorders and co-occurring trauma-related disorders to concurrent participation in mutual-help groups if appropriate ([Ouimette & Brown, 2003](#)).
- Counselors have a responsibility to practice the principles of confidentiality in all interactions with clients and to respect clients' wishes not to give up their right to privileged communication.
- Counselors are responsible for educating clients about the limits of confidentiality and what happens to protected health information, along with the client's privilege, when the client signs a release of information or agrees to assign insurance benefits to the provider.
- Counselors must inform clients that treatment for trauma-related disorders is always voluntary.
- Counselors are responsible for being aware of their own secondary trauma and countertransference reactions and seeking appropriate help in responding to these reactions so that they do not interfere with the best possible treatment for clients.

TIC organizations have responsibilities to clients in their care, including:

- Protecting client confidentiality, particularly in relation to clients' trauma histories. Organizations should comply with the State and Federal laws that protect the confidentiality of clients being treated for mental and substance use disorders.
- Providing clients with an easy-to-read statement of their rights as consumers of mental health and substance abuse services, including the right to confidentiality (Exhibit 2.2-4).
- Providing quality clinical supervision to all counselors and direct-service workers, with an emphasis on TIC. Organizations should, at minimum, comply with State licensing requirements for the provision of clinical supervision to behavioral health workers.
- Establishing and maintaining appropriate guidelines and boundaries for client and counselor behavior in the program setting.
- Creating and maintaining a trauma-informed treatment environment that respects the clients' right to self-determination and need to be treated with dignity and respect.
- Maintaining a work environment that reinforces and supports counselor self-care.

All behavioral health professionals are responsible for abiding by professional standards of care that protect the client. Breaches of confidentiality, inappropriate conduct, and other violations of trust can do further harm to clients who already have histories of trauma. Many treatment facilities have a Client Bill of Rights (or a similar document) that describes the rights and responsibilities of both the counselors and the participants; it often is part of the orientation and informed consent process when a client enters treatment. However, simply reading and acknowledging the receipt of a piece of paper is not a substitute for the dialog that needs to happen in a collaborative therapeutic partnership. Administrators are responsible for providing clients with easy-to-read information describing counselor responsibilities and client rights. Clinical supervisors are responsible for helping counselors engage in a respectful dialog with clients about those rights and responsibilities as part of a comprehensive informed consent process.

Exhibit 2.2-4 is an excerpt from a Client Bill of Rights that outlines clients' right to confidentiality in plain language that is readable and easily understood.

Exhibit 2.2-4

Sample Statement of the Client's Right to Confidentiality From a Client Bill of Rights

Tri-County Mental Health Services is a trauma-informed mental health and substance abuse treatment agency in Maine. Below is a statement regarding clients' right to confidentiality and staff responsibility to protect that privilege; this statement is provided in a brochure outlining consumer rights that is easily accessible to service recipients at the agency and online.

Confidentiality

We will not give out information about you to anyone without your knowledge and permission. This includes written information from your record and verbal information from your providers. Additionally, we will not request any information about you without your knowledge and permission. A Release of Information Form allows you to say what information can be shared and with whom. You determine the length of time this is valid, up to one year.

Tri-County policies prevent any employee of the agency who does not have a direct need to know from having access to any information about you. The penalty for violation can include immediate dismissal.

Exceptions to this rule of confidentiality include times when a client is at immediate risk of harm to self or others, or when ordered by the court. We will make every effort to notify you in these instances.

Source: [Tri-County Mental Health Services, 2008](#), pp. 6–7.

Ethics in Treating Traumatized Clients

All behavioral health professionals must conform to the ethical guidelines established by their profession's State licensing boards and/or certifying organizations. State licensing boards for substance abuse counseling, psychiatry, social work, psychology, professional counseling, and other behavioral health professions provide regulatory standards for ethical practice in these professions. These boards also have specific procedures for responding to complaints regarding the actions of professional caregivers. Additionally, national professional societies have standards for ethical practices. Members of these organizations are expected to practice within the boundaries and scope of these standards. Some of these standards are quite explicit, whereas others are more general; most approach professional ethics not as a rigid set of rules, but rather, as a process of making ethical decisions.

Clinical supervisors are responsible for informing counselors of their ethical responsibilities with regard to their own organization's policies and procedures, monitoring supervisees' reading and understanding the codes of ethics of professional organizations and State licensing boards, and promoting counselor understanding of ethics and how to make decisions ethically as a regular part of clinical supervision, team meetings, and counselor training. Administrators can support high ethical standards by creating an organization-wide ethics task group consisting of counselors, supervisors, and administrators who meet regularly to review and revise clinical policies in line with State and Federal law and professional codes of ethics. Administrators may also act as a support mechanism for counselors who need additional consultation regarding potential ethical dilemmas with clients. The Green Cross Academy of Traumatology provides ethical guidelines for the treatment of clients who have experienced trauma; these guidelines are adapted in Exhibit 2.2-5.

Exhibit 2.2-5

Green Cross Academy of Traumatology Ethical Guidelines for the Treatment of Clients Who Have Been Traumatized

Respect for the dignity of clients

- Recognize and value the personal, social, spiritual, and cultural diversity present in society, without judgment. As a primary ethical commitment, make every effort to provide interventions with respect for the dignity of those served.

Responsible caring

- Take the utmost care to ensure that interventions do no harm.
- Have a commitment to the care of those served until the need for care ends or the responsibility for care is accepted by another qualified service provider.
- Support colleagues in their work and respond promptly to their requests for help.
- Recognize that service to survivors of trauma can exact a toll in stress on providers. Maintain vigilance for signs in self and colleagues of such stress effects, and accept that dedication to the service of others imposes an obligation to sufficient self-care to prevent impaired functioning.
- Engage in continuing education in the appropriate areas of trauma response. Remain current in the field and ensure that interventions meet current standards of care.

Integrity in relationships

- Clearly and accurately represent your training, competence, and credentials. Limit your practice to methods and problems for which you are appropriately trained and qualified. Readily refer to or consult with colleagues who have appropriate expertise; support requests for such referrals or consultations from clients.
- Maintain a commitment to confidentiality, ensuring that the rights of confidentiality and privacy are maintained for all clients.
- Do not provide professional services to people with whom you already have either emotional ties or extraneous relationships of responsibility. The one exception is in the event of an emergency in which no other qualified person is available.
- Refrain from entering other relationships with present or former clients, especially sexual relationships or relationships that normally entail accountability.
- Within agencies, ensure that confidentiality is consistent with organizational policies; explicitly inform individuals of the legal limits of confidentiality.

Responsibility to society

- Be committed to responding to the needs generated by traumatic events, not only at the individual level, but also at the level of community and community organizations in ways that are consistent with your qualifications, training, and competence.
- Recognize that professions exist by virtue of societal charters in expectation of their functioning as socially valuable resources. Seek to educate government agencies and consumer groups about your expertise, services, and standards; support efforts by these agencies and groups to ensure social benefit and consumer protection.
- If you become aware of activities of colleagues that may indicate ethical violations or impairment of functioning, seek first to resolve the matter through direct expression of concern and offers of help to those colleagues. Failing a satisfactory resolution in this manner, bring the matter to the attention of the officers of professional societies and of governments with jurisdiction over professional misconduct.

Clients' universal rights

All clients have the right to:

- Not be judged for any behaviors they used to cope, either at the time of the trauma or after the trauma.
- Be treated at all times with respect, dignity, and concern for their well-being.
- Refuse treatment, unless failure to receive treatment places them at risk of harm to self or others.
- Be regarded as collaborators in their own treatment plans.
- Provide their informed consent before receiving any treatment.
- Not be discriminated against based on race, culture, sex, religion, sexual orientation, socioeconomic status, disability, or age.
- Have promises kept, particularly regarding issues related to the treatment contract, role of counselor, and program rules and expectations.

Procedures for introducing clients to treatment

Obtain informed consent, providing clients with information on what they can expect while receiving professional services. In addition to general information provided to all new clients, clients presenting for treatment who have histories of trauma should also receive information on:

- The possible short-term and long-term effects of trauma treatment on the client and the client's relationships with others.
- The amount of distress typically experienced with any particular trauma treatment.
- Possible negative effects of a particular trauma treatment.
- The possibility of lapses and relapses when doing trauma work, and the fact that these are a normal and expected part of healing.

Reaching counseling goals through consensus

Collaborate with clients in the design of a clearly defined contract that articulates a specific goal in a specific time period or a contract that allows for a more open-ended process with periodic evaluations of progress and goals.

Informing clients about the healing process

- Clearly explain to clients the nature of the healing process, making sure clients understand.
- Encourage clients to ask questions about any and all aspects of treatment and the therapeutic relationship. Provide clients with answers in a manner they can understand.
- Encourage clients to inform you if the material discussed becomes overwhelming or intolerable.
- Inform clients of the necessity of contacting you or emergency services if they feel suicidal or homicidal, are at risk of self-injury, or have a sense of being out of touch with reality.
- Give clients written contact information about available crisis or emergency services.
- Inform clients about what constitutes growth and recovery and about the fact that some trauma symptoms may not be fully treatable.
- Address unrealistic expectations clients may have about counseling and/or the recovery process.

Level of functioning

- Inform clients that they may not be able to function at the highest level of their ability—or even at their usual level—when working with traumatic material.
- Prepare clients to experience trauma-related symptoms, such as intrusive memories, dissociative reactions, reexperiencing, avoidance behaviors, hypervigilance, or unusual emotional reactivity.

Source: [Green Cross Academy of Traumatology, 2007](#). Adapted with permission.

Boundaries in therapeutic relationships

Maintaining appropriate therapeutic boundaries is a primary ethical concern for behavioral health professionals. Counselors working with clients who have substance use, trauma-related, and other mental disorders may feel challenged at times to maintain boundaries that create a safe therapeutic container. Some clients, especially those with longstanding disorders, bring a history of client–counselor relationships to counseling. Clients who have been traumatized may need help understanding the roles and responsibilities of both the counselor and the client. Clients with trauma-related conditions may also have special needs in establishing appropriate boundaries in the counseling setting; they may be particularly vulnerable and not understand or appreciate the need for professional boundaries, including not engaging in dual relationships. For example, some clients might experience a counselor's boundary around not giving the client his or her personal phone number for emergency calls as a rejection or abandonment. Cultural considerations also influence therapeutic boundaries.

[Advice to Clinical Supervisors: Recognizing Boundary Confusion](#)

Clinical supervisors should be aware of the following counselor behaviors that can indicate boundary confusion with clients:

- The counselor feels reluctant or embarrassed to discuss specific interactions with a client or details of the client's treatment in supervision or team meetings.
- The counselor feels possessive of the client, advocates with unusual and excessive vehemence for the client, or expresses an unreasonable sense of over responsibility for the client.
- The counselor becomes defensive and closed to hearing ideas from the supervisor or the treatment team members about approaches to working with a client and/or exploring his or her own emotional reactions to a client.
- The clinician begins or increases personal self-disclosure to the client and is not able to identify legitimate clinical reasons for the self-disclosure.

Administrators, in collaboration with clinical supervisors, are responsible for creating policies regarding counselor and client boundaries for various issues (e.g., giving and receiving gifts, counselor personal disclosure, and counselor roles and responsibilities when attending the same 12-Step meetings as clients); policies should be specific to their organization and conform to State and Federal law and behavioral health professional codes of ethics. Clinical supervisors are responsible for training counselors in the informed consent process and effective ways to discuss boundaries with clients when they enter treatment.

Guidelines for establishing and maintaining boundaries in therapeutic relationships, adapted from the Green Cross Academy of Traumatology, are given in [Exhibit 2.2-6](#).

[Exhibit 2.2-6](#)

[Boundaries in Therapeutic Relationships](#)

[Procedures for Establishing Safety](#)

Roles and boundaries

Counselor roles and boundaries should be established at the start of the counseling relationship and reinforced periodically, particularly at times when the client is experiencing high stress.

[Ongoing Relationships and the Issue of Boundaries](#)

Dual relationships

Dual relationships and inappropriate interactions with clients are to be avoided. It is important to tell clients at the beginning of counseling that contact between the counselor and the client can only occur within the boundaries of the professional relationship. This information is part of the informed consent process. Relationships outside these boundaries include sexual or romantic relationships, a counselor also serving as a client's sponsor in 12-Step programs, and any kind of relationship in which the counselor exploits the client for financial gain.

Sexual contact

- Never engage in any form of sexual contact with clients.
- Do not reward sexualized behaviors with attention or reactivity.
- Directly clarify the boundaries of the therapeutic relationship, and address the underlying motivations of persisting sexualized behavior.
- Set limits on a client's inappropriate behaviors while maintaining an ethos of care. Maintain respect for the dignity and worth of the client at all times.
- Understand that a client's attempt to sexualize a therapeutic relationship may reflect an early history of abuse, difficulty understanding social norms, or a variety of psychological problems.
- Readdress the absolute inappropriateness of sexual and/or romantic behavior in a nonlecturing, nonpunitive manner.
- If sexual behavior between clients occurs in a treatment program, counselors should consult with a clinical supervisor. Document the nature of the contact and how the issue is addressed.
- If a counselor has sexual contact with a client, he or she should take responsibility by ceasing counseling practice, referring clients to other treatment providers, and notifying legal and professional authorities. If a counselor is at risk for engaging with a client sexually but has not acted on it, the counselor should immediately consult with a supervisor, colleague, or psychotherapist.

Boundaries

Counselors should use care with self-disclosure or any behaviors that may be experienced as intrusive by the client, including:

- Personal disclosures made for the counselor's own gratification.
- Sexualized behavior with the client.
- Excessively intrusive questions or statements.
- Interrupting the client frequently.
- Violating the client's personal space.
- Interpersonal touch, which might activate intrusive memories or dissociative reactions or be experienced as a boundary violation by the client.
- Being consistently late for appointments or allowing outside influences (such as telephone calls) to interrupt the client's time in a counseling session.

Source: [Green Cross Academy of Traumatology, 2007](#). Adapted with permission.

[1] Clients with trauma histories may be especially vulnerable to counselor behaviors that are inconsistent or that are experienced by the client as boundary violations. Examples of such behavior include: being late for appointments, ending counseling sessions early, repeatedly and excessively extending the session time, canceling or "forgetting" appointments multiple times, spending time in the session talking about their own needs and life

experiences, exploring opportunities for contact outside the therapeutic relationship (including making arrangements to meet at AA or other 12-Step recovery group meetings), and enforcing rules differently for one client than for another.

Due to the complex dynamics that can arise in the treatment of clients with trauma histories, regularly scheduled clinical supervision, where issues of ethics and boundaries can be discussed, is recommended for counselors. For more information on how clinical supervision can be effectively used, see TIP 52, *Clinical Supervision and the Professional Development of the Substance Abuse Counselor* (CSAT, 2009b).

Boundary crossing and boundary violation

Although guidelines and codes of ethics are useful tools in helping clinical supervisors and counselors understand the boundaries between counselors and clients, they are open to interpretation and are context-bound. Given these limitations, it is crucial to educate counselors in TIC settings regarding the boundary issues that may arise for clients who have been traumatized and to give counselors a conceptual framework for understanding the contextual nature of boundaries. For example, it would be useful for clinical supervisors to discuss with counselors the distinction between boundary crossings and boundary violations in clinical practice. Gutheil and Brodsky (2008) define boundary crossing as a departure from the customary norms of counseling practice in relation to psychological, physical, or social space “that are harmless, are nonexploitative, and may even support or advance the therapy” (p. 20). Examples of boundary crossings include taking phone calls from a client between sessions if the client is in crisis or telling a client a story about the counselor’s recovery from trauma (without offering specific personal information or graphic/detailed description of the trauma) with the intention of offering hope that it is possible to recover.

Gutheil and Brodsky (2008) define boundary violations as boundary crossings that are unwanted and dangerous and which exploit the client, stating that “some boundary crossings are inadvisable because of their intent (i.e., they are not done in the service of the patient’s well-being and growth, involve extra therapeutic gratification for the therapist) and/or their effect (i.e., they are not likely to benefit the patient and entail a significant risk of harming the patient)” (pp. 20–21). An example of a boundary violation would be when a counselor invites a client to attend the same AA meetings the counselor attends or shares drinking and drugging “war stories” for the counselor’s own gratification. Two key elements in understanding when a boundary crossing becomes a boundary violation are the intent of the counselor and the damaging effect on the client. Maintaining a standard of practice of nonexploitation of the client is the primary focus for clinical supervisors and counselors in determining when boundary crossings become boundary violations.

Context is also an important consideration in determining the acceptability of boundary crossings. For example, it may be acceptable for a counselor in a partial hospitalization program for serious mental illness to have a cup of coffee at the kitchen table with a resident, whereas for a counselor in an outpatient mental health program, having a cup of coffee with a client at the local coffee shop would be a much more questionable boundary crossing.

Case Illustration: Denise

Denise is a 40-year-old licensed professional counselor working in an inpatient eating disorder program. She has had extensive training in trauma and eating disorder counseling approaches and has been working as a clinician in mental health settings for 15 years. Denise is usually open to suggestions from her supervisor and other treatment team members about specific strategies to use with clients who have trauma histories and eating disorders. However, in the past week, her supervisor has noticed that she has become defensive in team meetings and individual supervision when discussing a recently admitted young adult who was beaten and raped by her boyfriend; subsequently, the client was diagnosed with PTSD and anorexia. When the clinical supervisor makes note of the change in Denise’s attitude and behavior in team meetings since this young woman was admitted, initially Denise becomes defensive, saying that the team just doesn’t understand this young woman and that the client has repeatedly told Denise, “You’re the only counselor I trust.”

The clinical supervisor recognizes that Denise may be experiencing secondary traumatization and boundary confusion due to working with this young woman and to the recent increase in the number of clients with co-occurring trauma-related disorders on her caseload. After further exploration, Denise reveals that her own daughter was raped at the same age as the young woman and that hearing her story has activated an STS reaction in Denise. Her way of coping has been to become overly responsible for and overprotective of the young woman. With the nonjudgmental support of her supervisor, Denise is able to gain perspective, recognize that this young woman is not her daughter, and reestablish boundaries with her that are appropriate to the inpatient treatment setting.

Clinical Supervision and Consultation

Organizational change toward a TIC model doesn’t happen in isolation. Ongoing support, supervision, and consultation are key ingredients that reinforce behavioral health professionals’ training in trauma-informed and trauma-specific counseling methods and ensure compliance with practice standards and consistency over time. Often, considerable energy and resources are spent on the transition to new clinical and programmatic approaches, but without long-range planning to support those changes over time. The new treatment approach fades quickly, making it hard to recognize and lessening its reliability.

Advice to Clinical Supervisors and Administrators: Adopting an Evidence-Based Model of Clinical Supervision and Training

Just as adopting evidence-based clinical practices in a trauma-informed organization is important in providing cost-effective and outcome-relevant services to clients, adopting an evidence-based model of clinical supervision and training clinical supervisors in that model can enhance the quality and effectiveness of clinical supervision for counselors. This will ultimately enhance client care.

One of the most commonly used and researched integrative models of supervision is the discrimination model, originally published by Janine Bernard in 1979 and since updated (Bernard & Goodyear, 2009). This model is considered a competence-based and social role model of supervision; it includes three areas of focus on counselor competencies (intervention, conceptualization, and personalization) and three possible supervisor roles (teacher, counselor, and consultant).

Counselor competencies:

- Intervention: The supervisor focuses on the supervisee's intervention skills and counseling strategies used with a particular client in a given session.
- Conceptualization: The supervisor focuses on how the supervisee understands what is happening in a session with the client.
- Personalization: The supervisor focuses on the personal style of the counselor and countertransference responses (i.e., personal reactions) of the counselor to the client.

Supervisor roles:

- Teacher: The supervisor teaches the supervisee specific counseling theory and skills and guides the supervisee in the use of specific counseling strategies in sessions with clients. The supervisor as teacher is generally task-oriented. The supervisor is more likely to act as a teacher with beginning counselors.
- Counselor: The supervisor does not act as the counselor's therapist, but helps the counselor reflect on his or her counseling style and personal reactions to specific clients. The supervisor as counselor is interpersonally sensitive and focuses on the process and relational aspects of counseling.
- Consultant: The supervisor is more of a guide, offering the supervisee advice on specific clinical situations. The supervisor as consultant invites the counselor to identify topics and set the agenda for the supervision. The supervisor is more likely to act as a consultant with more advanced counselors.

This model of supervision may be particularly useful in working with counselors in TIC settings, because the supervisor's response to the supervisee is flexible and specific to the supervisee's needs. In essence, it is a counselor-centered model of supervision in which the supervisor can meet the most relevant needs of the supervisee in any given moment.

For a review of other theories and methods of clinical supervision, refer to TIP 52, *Clinical Supervision and Professional Development of the Substance Abuse Counselor* (CSAT, 2009b).

Ongoing supervision and consultation supports the organizational message that TIC is the standard of practice. It normalizes secondary traumatization as a systemic issue (not the individual pathology of the counselor) and reinforces the need for counselor self-care to prevent and lessen the impact of secondary traumatization. Quality clinical supervision for direct care staff demonstrates the organization's commitment to implementing a fully integrated, trauma-informed system of care.

Supervision and Consultation

Historically, there was an administrative belief that counselors who had extensive clinical experience and training would naturally be the best clinical supervisors. However, research does not support this idea (Falender & Shafranske, 2004). Although a competent clinical supervisor needs to have an extensive clinical background in the treatment of substance use, trauma-related, and other mental disorders, it is also essential for any counselor moving into a supervisory role to have extensive training in the theory and practice of clinical supervision before taking on this role. In particular, clinical supervisors in trauma-informed behavioral health settings should be educated in how to perform clinical supervision (not just administrative supervision) of direct service staff and in the importance of providing continuous clinical supervision and support for staff members working with individuals affected by trauma. Clinical supervision in a TIC organization should focus on the following priorities:

- General case consultation
- Specialized consultation in specific and unusual cases
- Opportunities to process clients' traumatic material
- Boundaries in the therapeutic and supervisory relationship
- Assessment of secondary traumatization
- Counselor self-care and stress management
- Personal growth and professional development of the counselor

Case Illustration: Arlene

Arlene is a 50-year-old licensed substance abuse counselor who has a personal history of trauma, and she is actively engaged in her own recovery from trauma. She is an experienced counselor who has several years of training in trauma-informed and trauma-specific counseling practices. Her clinical supervisor, acting in the role of consultant, begins the supervision session by inviting her to set the agenda. Arlene brings up a clinical situation in which she feels stuck with a client who is acting out in her Seeking Safety group (for more information on Seeking Safety, see Najavits, 2002a).

Arlene reports that her client gets up suddenly and storms out of the group room two or three times during the session. The supervisor, acting in the role of the counselor and focusing on personalization, asks Arlene to reflect on the client's behavior and what feelings are activated in her in response to the client's anger. Arlene is able to identify her own experience of hyperarousal and then paralysis as a stress reaction related to her prior experience of domestic violence in her first marriage. The supervisor, acting in the role of teacher and focusing on conceptualization, reminds Arlene that her client is experiencing a "fight-or-flight" response to some experience in the group that reminds her of her own trauma experience. The supervisor then suggests to Arlene that her own reactions are normal responses to her previous history of trauma, and that when her client is angry, Arlene is not reexperiencing her own trauma but is being activated by the client's traumatic stress reaction to being in group. In this way, the supervisor highlights the parallel process of the client-counselor's stress reactions to a perceived threat based on prior trauma experiences.

The supervisor, acting again as a consultant and focusing on personalization this time, invites Arlene to reflect on the internal and external resources she might be able to bring to this situation that will help remind her to ground herself so she can lessen the impact of her stress reactions on her counseling strategy with this client. Arlene states that she can create a list of safe people in her life and place this list in her pocket before group. She can use this list as a touchstone to remind her that she is safe and has learned many recovery skills that can help her stay grounded, maintain her boundaries, and deal with her client's behavior. The clinical supervisor, acting as a consultant and now focusing on intervention, asks Arlene if she has some specific ideas about how she can address the client's behavior in group. Arlene and the clinical supervisor spend the remainder of the session discussing different options for addressing the client's behavior and helping her feel safer in group.

Supervision of counselors working with traumatized clients should be regularly scheduled, with identified goals and with a supervisor who is trained and experienced in working with trauma survivors. The styles and types of supervision and consultation may vary according to the kind of trauma work and its context. For instance, trauma counseling in a major natural disaster would require a different approach to supervision and consultation than would counseling adults who experienced childhood developmental trauma or counseling clients in an intensive early recovery treatment program using a manualized trauma-specific counseling protocol.

Competence-based clinical supervision is recommended for trauma-informed organizations. Competence-based clinical supervision models identify the knowledge and clinical skills each counselor needs to master, and they use targeted learning strategies and evaluation procedures, such as direct observation of counselor sessions with clients, individualized coaching, and performance-based feedback. Studies on competence-based supervision approaches have demonstrated that these models improve counselor treatment skills and proficiency (Martino et al., 2011).

Whichever model of clinical supervision an organization adopts, the key to successful trauma-informed clinical supervision is the recognition that interactions between the supervisor and the counselor may parallel those between the counselor and the client. Clinical supervisors need to recognize counselors' trauma reactions (whether they are primary or secondary to the work with survivors of trauma) and understand that a confrontational or punitive approach will be ineffective and likely retraumatize counselors.

Clinical supervisors should adopt a respectful and collaborative working relationship with counselors in which role expectations are clearly defined in an informed consent process similar to that used in the beginning of the counselor–client relationship and in which exploring the nature of boundaries in both client–counselor and counselor–supervisor relationships is standard practice. Clear role boundaries, performance expectations, open dialog, and supervisor transparency can go a long way toward creating a safe and respectful relationship container for the supervisor and supervisee and set the stage for a mutually enhancing, collaborative relationship. This respectful, collaborative supervisory relationship is the main source of training and professional growth for the counselor and for the provision of quality care to people with behavioral health disorders.

Secondary Traumatization

The demands of caregiving exact a price from behavioral health professionals that cannot be ignored; otherwise, they may become ineffective in their jobs or, worse, emotionally or psychologically impaired. In a study of Master's level licensed social workers, 15.2 percent of respondents to a survey reported STS as a result of indirect exposure to trauma material at a level that meets the diagnostic criteria for PTSD. This rate is almost twice the rate of PTSD in the general population. The author concluded that behavioral health professionals' experience of STS is a contributing factor in staff turnover and one reason why many behavioral health service professionals leave the field (Bride, 2007). Secondary traumatization of behavioral health workers is a significant organizational issue for clinical supervisors and administrators in substance abuse and mental health treatment programs to address.

STS is a trauma-related stress reaction and set of symptoms resulting from exposure to another individual's traumatic experiences rather than from exposure directly to a traumatic event.

To prevent or lessen the impact of secondary traumatization on behavioral health professionals, clinical supervisors and administrators need to understand secondary trauma from the ecological perspective described in Part 1, Chapter 1 of this TIP. The organization itself creates a social context with risk factors that can increase the likelihood of counselors experiencing STS reactions, but it also contains protective factors that can lessen the risk and impact of STS reactions on staff members. Organizations can lessen the impact of the risk factors associated with working in trauma-informed organizations by mixing caseloads to contain clients both with and without trauma-related issues, supporting ongoing counselor training, providing regular clinical supervision, recognizing counselors' efforts, and offering an empowering work environment in which counselors share in the responsibility of making decisions and can offer input into clinical and program policies that affect their work lives.

When organizations support their counselors in their work with clients who are traumatized, counselors can be more effective, more productive, and feel greater personal and professional satisfaction. In addition, counselors develop a sense of allegiance toward the organization, thus decreasing staff turnover. If organizations do not provide this support, counselors can become demoralized and have fewer emotional and psychological resources to manage the impact of clients' traumatic material and outward behavioral expressions of trauma on their own well-being. Providing counselors with the resources to help them build resilience and prevent feeling overwhelmed should be a high priority for administrators and clinical supervisors in TIC organizations.

Risk and Protective Factors Associated With Secondary Traumatization

Clinical and research literature on trauma describes a number of factors related to the development of secondary trauma reactions and psychological distress in behavioral health professionals across a wide range of practice settings, as well as individual and organizational factors that can prevent or lessen the impact of STS on staff. The risk and protective factors model of understanding secondary trauma is based on the ecological perspective outlined in Part 1, Chapter 1 of this TIP. The terms "compassion fatigue," "vicarious traumatization," "secondary traumatization," and "burnout" are used in the literature, sometimes interchangeably and sometimes as distinct constructs. As stated in the terminology portion of the "How This TIP Is Organized" section that precedes Part 1, Chapter 1, of this TIP, the term "secondary traumatization" refers to traumatic stress reactions and psychological distress from exposure to another individual's traumatic experiences; this term will be used throughout this section, although the studies cited may use other terms.

Advice to Clinical Supervisors: Recognizing Secondary Traumatization

Some counselor behaviors that demonstrate inconsistency to clients may be outward manifestations of secondary traumatization, and they should be discussed with counselors through a trauma-informed lens. It is imperative that clinical supervisors provide a non-judgmental, safe context in which counselors can discuss these behaviors without fear of reprisal or reprimand. Clinical supervisors should work collaboratively with supervisees to help them understand their behavior and engage in self-care activities that lessen the stress that may be contributing to these behaviors.

Risk factors

Individual risk factors that may contribute to the development of STS in behavioral health professionals include preexisting anxiety or mood disorders; a prior history of personal trauma; high caseloads of clients with trauma-related disorders; being younger in age and new to the field with little clinical experience or training in treating trauma-related conditions; unhealthy coping styles, including distancing and detachment from clients and co-workers; and a lack of tolerance for strong emotions (Newall & MacNeil, 2010). Other negative coping strategies include substance abuse, other addictive behaviors, a lack of recreational activities not related to work, and a lack of engagement with social support. A recent study of trauma nurses found that low use of support systems, use of substances, and a lack of hobbies were among the coping strategies that differed between nurses with and without STS (Von Rueden et al., 2010). Other researchers found that clinicians who engaged in negative coping strategies, such as alcohol and illicit drug use, were more likely to experience intrusive trauma symptoms (Way, Van Deusen, Martin, Applegate, & Janle, 2004).

Advice to Clinical Supervisors: Recognizing STS in Counselors Who Are In Recovery

For counselors who are in recovery from a substance use or mental disorder, the development of STS may be a potential relapse concern. As Burke, Carruth, and Prichard (2006) point out, “a return to drinking or illicit drug use as a strategy for dealing with secondary trauma reactions would have a profoundly detrimental effect on the recovering counselor” (p. 292). So too, secondary trauma may ignite the reappearance of depressive or anxiety symptoms associated with a previous mental disorder. Clinical supervisors can address these risk factors with counselors and support them in engaging with their own recovery support network (which might include a peer support group or an individual counselor) to develop a relapse prevention plan.

Numerous organizational factors can contribute to the development of STS in counselors who work with clients with trauma-related disorders. These risk factors include organizational constraints, such as lack of resources for clients, lack of clinical supervision for counselors, lack of support from colleagues, and lack of acknowledgment by the organizational culture that secondary traumatization exists and is a normal reaction of counselors to client trauma (Newall & MacNeil, 2010). In a study of 259 individuals providing mental health counseling services, counselors who spent more time in session with clients with trauma-related disorders reported higher levels of traumatic stress symptoms (Bober & Regehr, 2006). Counselors may be more at risk for developing secondary traumatization if the organization does not allow for balancing the distribution of trauma and nontrauma cases amongst staff members.

Protective factors

Much of the clinical and research literature focuses on individual factors that may lessen the impact of STS on behavioral health professionals, including male gender, being older, having more years of professional experience, having specialized training in trauma-informed and trauma-specific counseling practices, lacking a personal trauma history, exhibiting personal autonomy in the workplace, using positive personal coping styles, and possessing resilience or the ability to find meaning in stressful life events and to rebound from adversity (Sprang, Clark, & Whitt-Woosley, 2007). Some of these factors, like positive personal coping styles and the ability to find meaning in adversity, can be developed and enhanced through personal growth work, psychotherapy, engagement with spiritual practices and involvement in the spiritual community, and stress reduction strategies like mindfulness meditation. A recent multi-method study of an 8-week workplace mindfulness training group for social workers and other social service workers found that mindfulness meditation increased coping strategies, reduced stress, and enhanced self-care of the participants; findings suggested that workers were more likely to practice stress management techniques like mindfulness at their place of work than at home (McGarrigle & Walsh, 2011). Organizations can support counselors’ individual efforts to enhance positive personal coping styles, find meaning in adversity, and reduce stress by providing time for workers during the workday for personal self-care activities, like mindfulness meditation and other stress reduction practices.

One of the organizational protective factors identified in the literature that may lessen the negative impact of secondary traumatization on behavioral health professionals is providing adequate training in trauma-specific counseling strategies, which increases providers’ sense of efficacy in helping clients with trauma-related disorders and reduces the sense of hopelessness that is often a part of the work (Bober & Regehr 2006). One study found that specialized trauma training enhanced job satisfaction and reduced levels of compassion fatigue, suggesting that “knowledge and training might provide some protection against the deleterious effects of trauma exposure” (Sprang et al., 2007, p. 272). Another protective factor that may lessen the chances of developing secondary traumatization is having a diverse caseload of clients. Organizations “must determine ways of distributing workload in order to limit the traumatic exposure of any one worker. This may not only serve to reduce the impact of immediate symptoms but may also address the potential longitudinal effects” (Bober & Regehr, 2006, p. 8).

Emotional support from professional colleagues can be a protective factor. A study of substance abuse counselors working with clients who were HIV positive found that workplace support from colleagues and supervisors most effectively prevented burnout (Shoptaw, Stein, & Rawson, 2000). This support was associated with less emotional fatigue and depersonalization, along with a sense of greater personal accomplishment. In a study of domestic violence advocates, workers who received more support from professional peers were less likely to experience secondary traumatization (Slattery & Goodman, 2009).

In addition, counselor engagement in relationally based clinical supervision with a trauma-informed supervisor acts as a protective agent. Slattery and Goodman (2009) note that “for the trauma worker, good supervision can normalize the feelings and experiences, provide support and information about the nature and course of the traumatic reaction, help in the identification of transference and countertransference issues, and reveal feelings or symptoms associated with the trauma” (p. 1362). Workers who reported “engaging, authentic, and empowering relationships with their supervisors” were less likely to experience STS (p. 1369). Thus, it is not simply the frequency and regularity of clinical supervision, but also the quality of the supervision and the quality of the supervisor–counselor relationship that can lessen the impact of STS on behavioral health professionals.

Engagement with a personal practice of spirituality that provides a sense of connection to a larger perspective and meaning in life is another protective factor that can lessen the impact of STS on counselors (Trippany, Kress, & Wilcoxon, 2004). Although recovering counselors may look to support groups for connection to a spiritual community, other behavioral health professionals might find support for enhancing spiritual meaning and connection in church, a meditation group, creative endeavors, or even volunteer work. The key is for counselors to develop their own unique resources and practices to

enhance a sense of meaningful spirituality in their lives. Clinical supervisors should be aware of spiritual engagement as a protective factor in preventing and lessening the impact of STS and should support clinicians in including it in their self-care plans, but they should take care not to promote or reject any particular religious belief system or spiritual practice.

Another protective factor that may lessen the impact of workers' STS is a culture of empowerment in the organization that offers counselors a sense of autonomy, a greater ability to participate in making decisions about clinical and organizational policies, and obtaining support and resources that further their professional development. Slattery & Goodman (2009) surveyed 148 domestic violence advocates working in a range of settings. The authors found that those workers "who reported a high level of shared power were less likely to report posttraumatic stress symptoms, despite their own personal abuse history or degree of exposure to trauma" (p. 1370). To the degree that organizations can provide a cultural context within which behavioral health professionals have autonomy and feel empowered, they will be able to lessen the impact of STS on their professional and personal lives. Self-efficacy and empowerment are antidotes to the experience of powerlessness that often accompanies trauma.

Strategies for Preventing Secondary Traumatization

The key to prevention of secondary traumatization for behavioral health professionals in a trauma-informed organization is to reduce risk and enhance protective factors. Organizational strategies to prevent secondary traumatization include:

- Normalize STS throughout all levels of the organization as a way to help counselors feel safe and respected, enhancing the likelihood that they will talk openly about their experiences in team meetings, peer supervision, and clinical supervision.
- Implement clinical workload policies and practices that maintain reasonable standards for direct-care hours and emphasize balancing trauma-related and nontrauma-related counselor caseloads.
- Increase the availability of opportunities for supportive professional relationships by promoting activities such as team meetings, peer supervision groups, staff retreats, and counselor training that focuses on understanding secondary traumatization and self-care. Administrators and clinical supervisors should provide time at work for counselors to engage in these activities.
- Provide regular trauma-informed clinical supervision that is relationally based. Supervisors should be experienced and trained in trauma-informed and trauma-specific practices and provide a competence-based model of clinical supervision that promotes counselors' professional and personal development. Supervision limited to case consultation or case management is insufficient to reduce the risk for secondary traumatization and promote counselor resilience.
- Provide opportunities for behavioral health professionals to enhance their sense of autonomy and feel empowered within the organization. Some of these activities include soliciting input from counselors on clinical and administrative policies that affect their work lives, including how to best balance caseloads of clients with and without histories of trauma; inviting representatives of the counseling staff to attend selected agency board of directors and/or management team meetings to offer input on workforce development; and inviting counselors to participate in organizational task forces that develop trauma-informed services, plan staff retreats, or create mechanisms to discuss self-care in team meetings. Administrators and clinical supervisors should assess the organization's unique culture and develop avenues for counselor participation in activities that will enhance their sense of empowerment and efficacy within the organization.

Exhibit 2.2-7 highlights some specific strategies that individual counselors can engage in to prevent secondary traumatization.

Exhibit 2.2-7

Counselor Strategies To Prevent Secondary Traumatization

- Strategies that counselors can use (with the support and encouragement of supervisors and administrators) to prevent secondary traumatization include:
 - Peer support: Maintaining adequate social support, both personally and professionally, helps prevent isolation and helps counselors share the emotional distress of working with traumatized individuals.
 - Supervision and consultation: Professional consultation will help counselors understand secondary traumatization, their own personal risks, the protective factors that can help them prevent or lessen its impact, and their countertransference reactions to specific clients.
- Training: Ongoing professional training can improve counselors' understanding of trauma and enhance a sense of mastery and self-efficacy in their work.
- Personal psychotherapy or counseling: Being in counseling can help counselors become more self-aware and assist them in managing the psychological and emotional distress that often accompanies working with clients who have trauma histories in a number of behavioral health settings.
- Maintaining balance in one's life: Balancing work and personal life, developing positive coping styles, and maintaining a healthy lifestyle can enhance resilience and the ability to manage stress.
- Engaging in spiritual activities that provide meaning and perspective: Connection to a spiritual community and spiritual practices (such as meditation) can help counselors gain a larger perspective on trauma and enhance resilience.

Assessment of Secondary Traumatization

Counselors with unacknowledged STS can harm clients, self, and family and friends by becoming unable to focus on and attend to their needs or those of others. They may feel helpless or cynical and withdraw from support systems. Exhibit 2.2-8 describes some emotional, cognitive, and behavioral signs that may indicate that a counselor is experiencing secondary traumatization. Clinical supervisors should be familiar with the manifestations of STS in their counselors and should address signs of STS immediately.

Exhibit 2.2-8 Secondary Traumatization Signs

The following are some indicators that counselors may be experiencing secondary traumatization.

Psychological distress

- Distressing emotions: grief, depression, anxiety, dread, fear, rage, shame
- Intrusive imagery of client's traumatic material: nightmares, flooding, flashbacks of client disclosures
- Numbing or avoidance: avoidance of working with client's traumatic material
- Somatic issues: sleep disturbances, headaches, gastrointestinal distress, heart palpitations, chronic physiological arousal
- Addictive/compulsive behaviors: substance abuse, compulsive eating, compulsive working
- Impaired functioning: missed or canceled appointments, decreased use of supervision, decreased ability to engage in self-care, isolation and alienation

Cognitive shifts

- Chronic suspicion about others
- Heightened sense of vulnerability
- Extreme sense of helplessness or exaggerated sense of control over others or situations
- Loss of personal control or freedom
- Bitterness or cynicism
- Blaming the victim or seeing everyone as a victim
- Witness or clinician guilt if client reexperiences trauma or reenacts trauma in counseling
- Feeling victimized by client

Relational disturbances

- Decreased intimacy and trust in personal/professional relationships
- Distancing or detachment from client, which may include labeling clients, pathologizing them, judging them, canceling appointments, or avoiding exploring traumatic material
- Over identification with the client, which may include a sense of being paralyzed by one's own responses to the client's traumatic material or becoming overly responsible for the client's life

Frame of reference

- Disconnection from one's sense of identity
- Dramatic change in fundamental beliefs about the world
- Loss or distortion of values or principles
- A previous sense of spirituality as comfort or resource decreases or becomes nonexistent
- Loss of faith in something greater
- Existential despair and loneliness

Sources: Figley, 1995; Newall & MacNeil, 2010; Saakvitne et al., 1996.

Stamm (2009–2012) has developed and revised a self-assessment tool, the Professional Quality of Life Scale (ProQOL), that measures indicators of counselor compassion fatigue and compassion satisfaction. Compassion fatigue “is best defined as a syndrome consisting of a combination of the symptoms of secondary traumatic stress and professional burnout” (Newall & MacNeil, 2010, p. 61). Although secondary traumatization as a reaction to exposure to clients' trauma material is similar to PTSD, burnout is a more general type of psychological distress related to the pressures of working in high-stress environments over time. Burnout may be a result of secondary traumatization and/or a contributing factor in the development of secondary traumatization. The ProQOL includes STS and burnout scales that have been validated in research studies (Adams, Figley, & Boscarino, 2008; Newall & MacNeil, 2010).

This tool can be used in individual and group clinical supervision, trainings on self-care, and team meetings as a way for counselors to check in with themselves on their levels of stress and potential signs of secondary traumatization.

Case Illustration: Gui

Gui is a 48-year-old licensed substance abuse counselor who has worked in a methadone maintenance clinic for 12 years. He originally decided to get his degree and become a counselor because he wanted to help people and make a difference in the world. Over the past 6 months, he has felt fatigued a great deal, gets annoyed easily with both clients and coworkers, and has developed a cynical attitude about the world and the people who come to the clinic for help. During this time, the clinic has been forced to lay off a number of counselors due to funding cutbacks. As a result, Gui and the remaining counselors have had a 20 percent increase in the number of weekly client contact hours required as part of their job duties. In addition, the level and severity of clients' trauma-related and other co-occurring disorders, poverty, joblessness, and homelessness has increased.

Gui is a valued employee, and when Gui discusses his thoughts that he might want to leave the clinic with his clinical supervisor, the supervisor listens to Gui's concerns and explores the possibility of having him fill out the ProQOL to get a pulse on his stress level. Gui agrees and is willing to discuss the results with his supervisor. He is not surprised to see that he scores above average on the burnout scale of the instrument but is very surprised to see that he scores below average on the secondary traumatic stress scale and above average on the compassion satisfaction scale. He begins to feel more hopeful that he still finds satisfaction in his job and sees that he is resilient in many ways that he did not acknowledge before.

Gui and the clinical supervisor discuss ways that the supervisor and the organization can lessen the impact of the stress of the work environment on Gui and support the development of a self-care plan that emphasizes his own ability to rebound from adversity and take charge of his self-care.

The compassion satisfaction scale allows counselors to reflect on their resilience and reminds them of why they choose to work with people with substance use and trauma-related disorders, despite the fact that this work can lead to secondary traumatization. The compassion satisfaction subscale reminds counselors that they are compassionate, that one of the reasons they are in a helping profession is that they value service to others, and that helping brings meaning and fulfillment to their lives. Exhibits 2.2-9 through 2.2-11 present the most recent version of the ProQOL.

Exhibit 2.2-9 PRoQOL Scale

COMPASSION SATISFACTION AND COMPASSION FATIGUE (PRoQOL) VERSION 5 (2009)

When you [help] people you have direct contact with their lives. As you may have found, your compassion for those you [help] can affect you in positive and negative ways. Below are some questions about your experiences, both positive and negative, as a [helper]. Consider each of the following questions about you and your current work situation. Select the number that honestly reflects how frequently you experienced these things in the *past 30 days*.

1=Never 2=Rarely 3=Sometimes 4=Often 5=Very Often

- ___ 1. I am happy.
- ___ 2. I am preoccupied with more than one person I [help].
- ___ 3. I get satisfaction from being able to [help] people.
- ___ 4. I feel connected to others.
- ___ 5. I jump or am startled by unexpected sounds.
- ___ 6. I feel invigorated after working with those I [help].
- ___ 7. I find it difficult to separate my personal life from my life as a [helper].
- ___ 8. I am not as productive at work because I am losing sleep over traumatic experiences of a person I [help].
- ___ 9. I think that I might have been affected by the traumatic stress of those I [help].
- ___ 10. I feel trapped by my job as a [helper].
- ___ 11. Because of my [helping], I have felt “on edge” about various things.
- ___ 12. I like my work as a [helper].
- ___ 13. I feel depressed because of the traumatic experiences of the people I [help].
- ___ 14. I feel as though I am experiencing the trauma of someone I have [helped].
- ___ 15. I have beliefs that sustain me.
- ___ 16. I am pleased with how I am able to keep up with [helping] techniques and protocols.
- ___ 17. I am the person I always wanted to be.
- ___ 18. My work makes me feel satisfied.
- ___ 19. I feel worn out because of my work as a [helper].
- ___ 20. I have happy thoughts and feelings about those I [help] and how I could help them.
- ___ 21. I feel overwhelmed because my case [work] load seems endless.
- ___ 22. I believe I can make a difference through my work.
- ___ 23. I avoid certain activities or situations because they remind me of frightening experiences of the people I [help].
- ___ 24. I am proud of what I can do to [help].
- ___ 25. As a result of my [helping], I have intrusive, frightening thoughts.
- ___ 26. I feel “bogged down” by the system.
- ___ 27. I have thoughts that I am a “success” as a [helper].
- ___ 28. I can’t recall important parts of my work with trauma victims.
- ___ 29. I am a very caring person.
- ___ 30. I am happy that I chose to do this work.

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Source: Stamm, 2012. Used with permission.

Exhibit 2.2-10 Your Scores on the ProQOL: Professional Quality of Life Screening

Based on your responses, place your personal scores below. If you have any concerns, you should discuss them with a physical or mental healthcare professional.

Compassion Satisfaction _____

Compassion satisfaction is about the pleasure you derive from being able to do your work well. For example, you may feel like it is a pleasure to help others through your work. You may feel positively about your colleagues or your ability to contribute to the work setting or even the greater good of society. Higher scores on this scale represent a greater satisfaction related to your ability to be an effective caregiver in your job.

The average score is 50 (SD 10; alpha scale reliability .88). About 25% of people score higher than 57 and about 25% of people score below 43. If you are in the higher range, you probably derive a good deal of professional satisfaction from your position. If your scores are below 40, you may either find problems with your job, or there may be some other reason—for example, you might derive your satisfaction from activities other than your job.

Burnout

Most people have an intuitive idea of what burnout is. From the research perspective, burnout is one of the elements of Compassion Fatigue (CF). It is associated with feelings of hopelessness and difficulties in dealing with work or in doing your job effectively. These negative feelings usually have a gradual onset. They can reflect the feeling that your efforts make no difference, or they can be associated with a very high workload or a non-supportive work environment. Higher scores on this scale mean that you are at higher risk for burnout.

The average score on the burnout scale is 50 (SD 10; alpha scale reliability .75). About 25% of people score above 57 and about 25% of people score below 43. If your score is below 43, this probably reflects positive feelings about your ability to be effective in your work. If you score above 57 you may wish to think about what at work makes you feel like you are not effective in your position. Your score may reflect your mood; perhaps you were having a “bad day” or are in need of some time off. If the high score persists or if it is reflective of other worries, it may be a cause for concern.

Secondary Traumatic Stress

The second component of Compassion Fatigue (CF) is secondary traumatic stress (STS). It is about your work related, secondary exposure to extremely or traumatically stressful events. Developing problems due to exposure to other’s trauma is somewhat rare but does happen to many people who care for those who have experienced extremely or traumatically stressful events. For example, you may repeatedly hear stories about the traumatic things that happen to other people, commonly called Vicarious Traumatization. If your work puts you directly in the path of danger, for example, field work in a war or area of civil violence, this is not secondary exposure; your exposure is primary. However, if you are exposed to others’ traumatic events as a result of your work, for example, as a therapist or an emergency worker, this is secondary exposure. The symptoms of STS are usually rapid in onset and associated with a particular event. They may include being afraid, having difficulty sleeping, having images of the upsetting event pop into your mind, or avoiding things that remind you of the event.

The average score on this scale is 50 (SD 10; alpha scale reliability .81). About 25% of people score below 43 and about 25% of people score above 57. If your score is above 57, you may want to take some time to think about what at work may be frightening to you or if there is some other reason for the elevated score. While higher scores do not mean that you do have a problem, they are an indication that you may want to examine how you feel about your work and your work environment. You may wish to discuss this with your supervisor, a colleague, or a healthcare professional.

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Source: [Stamm, 2012](#). Used with permission.

Exhibit 2.2-11

Addressing Secondary Traumatization

If a counselor is experiencing STS, the organization should address it immediately. Clinical supervisors can collaborate with counselors to devise an individualized plan that is accessible, acceptable, and appropriate for each counselor and that addresses the secondary stress reactions the counselor is experiencing, providing specific self-care strategies to counteract the stress. Decisions about strategies for addressing secondary traumatization should be based on the personal preferences of the counselor, the opportunity for an immediate intervention following a critical incident, and the counselor’s level of awareness regarding his or her experience of STS. Counselors may need to talk about what they are experiencing, feeling, and thinking. These experiences can be processed in teams, in consultations with colleagues, and in debriefing meetings to integrate them effectively ([Myers & Wee, 2002](#)).

If a critical incident evokes secondary traumatization among staff—such as a client suicide, a violent assault in the treatment program, or another serious event—crisis intervention should be available for workers who would like to participate. Any intervention should be voluntary and tailored to each worker’s individual needs (e.g., peer, group, or individual sessions); if possible, these services should be offered continuously instead of just one time.

The objective of debriefing a critical incident that evokes STS reactions in counselors is to help them dissipate the hyperarousal associated with traumatic stress and prevent long-term aftereffects that might eventually lead to counselor impairment. Because clinical supervisors may also be experiencing secondary traumatization, it is advisable for administrators to invite an outside trauma consultant into the organization to provide a safe space for all staff members (including clinical supervisors) to address and process the critical stress incident. For noncrisis situations, secondary traumatization should be addressed in clinical supervision. Clinical supervisors and counselors should work collaboratively to incorporate regular screening and self-assessment of STS into supervision sessions.

Advice to Clinical Supervisors: Advantages and Disadvantages of Using Psychometric Measures

Using a psychometric measure such as the ProQOL has advantages and disadvantages. It is important to understand that all tests measure averages and ranges but do not account for individual circumstances.

If you use the ProQOL in clinical supervision, present it as a self-assessment tool. Let counselors opt out of sharing their specific results with you and/or your team if it is administered in a group. If counselors choose to share scores on specific items or scales with you, work collaboratively and respectfully with them to explore their own understanding of and meanings attached to their scores. If this tool is not presented to supervisees in a nonjudgmental, mindful way, counselors may feel as if they have failed if their scores on the secondary traumatization scale are above average or if their scores on the compassion satisfaction scale are below average. High scores on the compassion fatigue and burnout scales do not mean that counselors don’t care about

their clients or that they aren't competent clinicians. The scores are simply one way for you and your supervisees to get a sense of whether they might be at risk for secondary traumatization, what they can do to prevent it, how to address it, and how you can support them.

The potential benefits of using a self-assessment tool like the ProQOL in clinical supervision are that it can help counselors:

- Reflect on their emotional reactions and behaviors and identify possible triggers for secondary traumatization.
- Assess their risk levels.
- Examine alternative coping strategies that may prevent secondary traumatization.
- Understand their own perceptions of themselves and their job satisfaction, affirming what they already know about their risk of secondary traumatization and their compassion satisfaction.
- Reflect on different factors that might contribute to unexpected low or high scores, such as the day of the week, the intensity of the workload, whether they have just come back from the weekend or a vacation, and so forth.
- Increase self-awareness and self-knowledge, because scores on specific items or scales bring to consciousness what is often outside of awareness.
- Realize how resilient they are emotionally, mentally, physically, and spiritually.
- Become aware of and open up conversations about self-care and self-care activities and resources, such as supportive coworkers, team members, and social networks outside of work.

If used regularly, self-assessment tools can help counselors and clinical supervisors monitor STS levels, indicate significant positive and negative changes, and suggest action toward self-care in specific areas. Clinical supervisors should fill out the ProQOL and review results with their own supervisors, a peer supervisor, or a colleague before administering it to supervisees. Doing so enables supervisors to gauge their own reactions to the self-assessment and anticipate potential reactions from supervisees.

Advice to Clinical Supervisors: Is it Supervision or Psychotherapy?

Although there are some aspects of clinical supervision that can be therapeutic and parallel the therapeutic and emotional support that occurs between the counselor and the client, clinical supervision is not therapy. As a result, it is important for clinical supervisors to maintain appropriate boundaries with supervisees when addressing their STS reactions at work.

When does the process in supervision cross over into the realm of practicing therapy with a supervisee? One clear indicator is if the supervisor begins to explore the personal history of the counselor and reflects directly on that history instead of bringing it back to how the counselor's history influences his or her work with a particular client or with clients with trauma histories in general. Clinical supervisors should focus only on counselor issues that may be directly affecting their clinical functioning with clients. If personal issues arise in clinical supervision, counselors should be encouraged to address them in their own counseling or psychotherapy.

When STS issues arise, the clinical supervisor should work with counselors to review and revise their self-care plans to determine what strategies are working and whether additional support, like individual psychotherapy or counseling, may be warranted.

Counselor Self-Care

In light of the intensity of therapeutic work with clients with co-occurring substance use, mental, and trauma-related disorders and the vulnerability of counselors to secondary traumatization, a comprehensive, individualized self-care plan is highly recommended. Balance is the key to the development of a self-care plan—a balance between home and work, a balance between focusing on self and others, and a balance between rest and activity (Saakvitne, Perlman, & Traumatic Stress Institute/Center for Adult & Adolescent Psychotherapy 1996). Counselor self-care is also about balancing vulnerability, which allows counselors to be present and available when clients address intensely painful content, with reasonable efforts to preserve their sense of integrity in situations that may threaten the counselors' faith or worldview (Burke et al., 2006). A comprehensive self-care plan should include activities that nourish the physical, psychological/mental, emotional/relational, and spiritual aspects of counselors' lives.

The literature on counselor self-care advocates for individual, team, and organizational strategies that support behavioral health professionals working with clients who have substance use and trauma-related disorders. Counselors are responsible for developing comprehensive self-care plans and committing to their plans, but clinical supervisors and administrators are responsible for promoting counselor self-care, supporting implementation of counselor self-care plans, and modeling self-care. Counselor self-care is an ethical imperative; just as the entire trauma-informed organization must commit to other ethical issues with regard to the delivery of services to clients with substance use, mental, and trauma-related disorders, it must also commit to the self-care of staff members who are at risk for secondary traumatization as an ethical concern. Saakvitne and colleagues (1996) suggest that when administrators support counselor-self-care, it is not only cost-effective in that it reduces the negative effects of secondary traumatization on counselors (and their clients), but also promotes "hope-sustaining behaviors" in counselors, making them more motivated and open to learning, and thereby improving job performance and client care.

Case Illustration: Carla

Carla is a 38-year-old case manager working in an integrated mental health and substance abuse agency. She provides in-home case management services to home-bound clients with chronic health and/or severe mental health and substance abuse problems. Many of her clients have PTSD and chronic, debilitating pain.

Both her parents had alcohol use disorders, and as a result, Carla became the caretaker in her family. She loves her job; however, she often works 50 to 60 hours per week and has difficulty leaving her work at work. She often dreams about her clients and wakes up early, feeling anxious. She sometimes has traumatic nightmares, even though she was never physically or sexually abused, and she has never experienced the trauma of violence or a natural disaster. She drinks five cups of coffee and three to four diet sodas every day and grabs burgers and sweets for snacks while she drives from one client to

the next. She has gained 20 pounds in the past year and has few friends outside of her coworkers. She has not taken a vacation in more than 2 years. She belongs to the Catholic church down the street, but she has stopped going because she says she is too busy and exhausted by the time Sunday rolls around.

The agency brings in a trainer who meets with the case management department and guides the staff through a self-assessment of their current self-care practices and the development of a comprehensive self-care plan. During the training, Carla acknowledges that she has let her work take over the rest of her life and needs to make some changes to bring her back into balance. She writes out her self-care plan, which includes cutting back on the caffeine, calling a friend she knows from church to go to a movie, going to Mass on Sunday, dusting off her treadmill, and planning a short vacation to the beach. She also decides that she will discuss her plan with her supervisor and begin to ask around for a counselor for herself to talk about her anxiety and her nightmares. In the next supervision session, Carla's supervisor reviews her self-care plan with her and helps Carla evaluate the effectiveness of her self-care strategies. Her supervisor also begins to make plans for how to cover Carla's cases when she takes her vacation.

A Comprehensive Self-Care Plan

A self-care plan should include a self-assessment of current coping skills and strategies and the development of a holistic, comprehensive self-care plan that addresses the following four domains:

1. Physical self-care
2. Psychological self-care (includes cognitive/mental aspects)
3. Emotional self-care (includes relational aspects)
4. Spiritual self-care

Activities that may help behavioral health workers find balance and cope with the stress of working with clients with trauma-related disorders include talking with colleagues about difficult clinical situations, attending workshops, participating in social activities with family and friends, exercising, limiting client sessions, balancing caseloads to include clients with and without trauma histories, making sure to take vacations, taking breaks during the workday, listening to music, walking in nature, and seeking emotional support in both their personal and professional lives (Saakvitne et al., 1996). In addition, regular clinical supervision and personal psychotherapy or counseling can be positive coping strategies for lessening the impact of STS on counselors. Still, each counselor is unique, and a self-care approach that is helpful to one counselor may not be helpful to another. Exhibits 2.2-13 and 2.2-14 offer tools for self-reflection to help counselors discover which specific self-care activities might best suit them. The worksheet can be used privately by counselors or by clinical supervisors as an exercise in individual supervision, group supervision, team meetings, or trainings on counselor self-care.

Advice to Clinical Supervisors: Spirituality

The word "spiritual" in this context is used broadly to denote finding a sense of meaning and purpose in life and/or a connection to something greater than the self. Spiritual meanings and faith experiences are highly individual and can be found within and outside of specific religious contexts.

Engaging in spiritual practices, creative endeavors, and group/community activities can foster a sense of meaning and connection that can counteract the harmful effects of loss of meaning, loss of faith in life, and cognitive shifts in worldview that can be part of secondary traumatization. Counselors whose clients have trauma-related disorders experience fewer disturbances in cognitive schemas regarding worldview and less hopelessness when they engage in spiritually oriented activities, such as meditation, mindfulness practices, being in nature, journaling, volunteer work, attending church, and finding a spiritual community (Burke et al., 2006). Clinical supervisors can encourage counselors to explore their own spirituality and spiritual resources by staying open and attuned to the multidimensional nature of spiritual meaning of supervisees and refraining from imposing any particular set of religious or spiritual beliefs on them. A strong sense of spiritual connection can enhance counselors' resilience and ability to cope with the sometimes overwhelming effects of clients' trauma material and trauma-related behavior (including suicidality) on counselors' faith in life and sense of meaning and purpose.

Modeling Self-Care

"Implementing interventions was not always easy, and one of the more difficult coping strategies to apply had to do with staff working long hours. Many of the staff working at the support center also had full-time jobs working for the Army. In addition, many staff chose to volunteer at the Family Assistance Center and worked 16- to 18-hour days. When we spoke with them about the importance of their own self-care, many barriers emerged: guilt over not working, worries about others being disappointed in them, fear of failure with respect to being unable to provide what the families might need, and a 'strong need to be there.' Talking with people about taking a break or time off proved problematic in that many of them insisted that time off was not needed, despite signs of fatigue, difficulty concentrating, and decreased productivity. Additionally, time off was not modeled. Management, not wanting to fail the families, continued to work long hours, despite our requests to do otherwise. Generally, individuals could see and understand the reasoning behind such endeavors. Actually making the commitment to do so, however, appeared to be an entirely different matter. In fact, our own team, although we kept reasonable hours (8 to 10 per day), did not take a day off in 27 days. Requiring time off as part of membership of a Disaster Response Team might be one way to solve this problem."

—Member of a Disaster Response Team at the Pentagon after September 11

Source: Walser, 2004, pp. 4–5.

Review the questions in Exhibit 2.2-14, and then write down specific self-care strategies in the form (given in Exhibit 2.2-13) that you're confident you will practice in both personal and professional realms.

The *Comprehensive Self-Care Worksheet* is a tool to help counselors (and clinical supervisors) develop awareness of their current coping strategies and where in the four domains they need to increase their engagement in self-care activities. Once completed, clinical supervisors should periodically review

the plan with their supervisees for effectiveness in preventing and/or ameliorating secondary traumatization and then make adjustments as needed.

Exhibit 2.2-13

Comprehensive Self-Care Plan Worksheet

Name:		
Date:	Personal	Professional/Workspace
Physical		
Psychological/Mental		
Emotional/Relational		
Spiritual		

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Exhibit 2.2-14

Comprehensive Self-Care Plan Worksheet Instructions

Use the following questions to help you engage in a self-reflective process and develop your comprehensive self-care plan. Be specific and include strategies that are accessible, acceptable, and appropriate to your unique circumstances. Remember to evaluate and revise your plan regularly.

Physical

- What are non-chemical things that help my body relax?
- What supports my body to be healthy?

Psychological/Mental

- What helps my mind relax?
- What helps me see a bigger perspective?
- What helps me break down big tasks into smaller steps?
- What helps me counteract negative self-talk?
- What helps me challenge negative beliefs?
- What helps me build my theoretical understanding of trauma and addictions?
- What helps me enhance my counseling/helping skills in working with traumatized clients?
- What helps me become more self-reflective?

Emotional/Relational

- What helps me feel grounded and able to tolerate strong feelings?
- What helps me express my feelings in a healthy way?
- Who helps me cope in positive ways and how do they help?
- What helps me feel connected to others?
- Who are at least three people I feel safe talking with about my reactions/feelings about clients?
- How can I connect with those people on a regular basis?

Spiritual

- What helps me find meaning in life?
- What helps me feel hopeful?
- What sustains me during difficult times?
- What connects me to something greater?

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Essential Components of Self-Care

Saakvitne and colleagues (1996) describe three essential components, the “ABCs,” of self-care that effectively address the negative impact of secondary traumatization on counselors:

1. **Awareness** of one’s needs, limits, feelings, and internal/external resources. Awareness involves mindful/nonjudgmental attention to one’s physical, psychological, emotional, and spiritual needs. Such attention requires quiet time and space that supports self-reflection.
2. **Balance** of activities at work, between work and play, between activity and rest, and between focusing on self and focusing on others. Balance provides stability and helps counselors be more grounded when stress levels are high.

3. **Connection** to oneself, to others, and to something greater than the self. Connection decreases isolation, increases hope, diffuses stress, and helps counselors share the burden of responsibility for client care. It provides an anchor that enhances counselors' ability to witness tremendous suffering without getting caught up in it.

Clinical supervisors can help counselors review their self-care plans through the ABCs by reflecting on these questions:

1. Has the counselor accurately identified his or her needs, limits, feelings, and internal and external resources in the four domains (physical, psychological/mental, emotional/relational, spiritual)?
2. Has the counselor described self-care activities that provide a balance between work and leisure, activity and rest, and a focus on self and others?
3. Has the counselor identified self-care activities that enhance connection to self, others, and something greater than self (or a larger perspective on life)?

Supervisors should make their own self-care plans and review them periodically with their clinical supervisors, a peer supervisor, or a colleague.

Commitment to Self-Care

One of the major obstacles to self-care is giving in to the endless demands of others, both at work and at home. It is therefore essential for counselors with the support of clinical supervisors to become “guardians of [their] boundaries and limits” (Saakvitne et al., 1996, p 136). Creating a daily schedule that includes breaks for rest, exercise, connection with coworkers, and other self-care activities can support counselors in recognizing that they are valuable individuals who are worthy of taking the time to nourish and nurture themselves, thus increasing commitment to self-care. Another way to support counselors in committing to self-care is for supervisors and administrators to model self-care in their own professional and personal lives.

Understanding that counselor self-care is not simply a luxury or a selfish activity, but rather, an ethical imperative (Exhibit 2.2-15) can foster counselors' sense of connection to their own values and accountability to the people they serve as competent and compassionate caregivers. Clinical supervisors and administrators can reinforce this sense of accountability while supporting counselors by providing a caring, trauma-informed work environment that acknowledges and normalizes secondary traumatization and by offering reasonable resources that make it possible for counselors to do their work and take care of themselves at the same time. Preventing secondary traumatization and lessening its impact on counselors once it occurs is not only cost-effective with regard to decreasing staff turnover and potential discontinuity of services to clients; it is also the ethical responsibility of a trauma-informed organization.

Exhibit 2.2-15 The Ethics of Self-Care

The Green Cross Academy of Traumatology was originally established to serve a need in Oklahoma City following the April 19, 1995, bombing of the Alfred P. Murrah Federal Building. Below are adapted examples of the Academy's code of ethics with regard to worker self-care.

Ethical Principles of Self-Care in Practice

These principles declare that it is unethical not to attend to your self-care as a practitioner, because sufficient self-care prevents harming those we serve.

Standards of self-care guidelines:

- Respect for the dignity and worth of self: A violation lowers your integrity and trust.
- Responsibility of self-care: Ultimately it is your responsibility to take care of yourself—and no situation or person can justify neglecting this duty.
- Self-care and duty to perform: There must be a recognition that the duty to perform as a helper cannot be fulfilled if there is not, at the same time, a duty to self-care.

Standards of humane practice of self-care:

- Universal right to wellness: Every helper, regardless of her or his role or employer, has a right to wellness associated with self-care.
- Physical rest and nourishment: Every helper deserves restful sleep and physical separation from work that sustains them in their work role.
- Emotional rest and nourishment: Every helper deserves emotional and spiritual renewal both in and outside the work context.
- Sustenance modulation: Every helper must utilize self-restraint with regard to what and how much they consume (e.g., food, drink, drugs, stimulation) since improper consumption can compromise their competence as a helper.

Commitment to self-care:

- Make a formal, tangible commitment: Written, public, specific, measurable promises of self-care.
- Set deadlines and goals: The self-care plan should set deadlines and goals connected to specific activities of self-care.
- Generate strategies that work and follow them: Such a plan must be attainable and followed with great commitment and monitored by advocates of your self-care.

Source: [Green Cross Academy of Traumatology, 2010](#). Adapted with permission.

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2.2: Ethics of Practice

<https://kspope.com/ethics/research4.php>

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2.3: Stress and Coping

Learning Objectives

- Understand what stress is and the different forms people can experience
- Understand what coping is, what coping strategies are, and coping styles
- Understand how individuals and communities become resilient



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WHAT IS STRESS?



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So what is **stress**? This shouldn't be a trick question, but why is it so hard to answer? Stress can be three things: a *stimulus event* (i.e., a stressor), a *process for understanding the stimulus and its context*, and a *reaction we have to this event*. Essentially, to be stressful the event has to become an overload of incoming information into our system. Stress can cause biological responses such as sweaty palms or a racing heart, as well as psychological responses such as nervousness. It is known to have effects on our behavior causing us to avoid others, and it also affects cognitive performance causing us to have difficulty concentrating.

Major Life Events (e.g., experiencing a breakup, getting married, or having a baby), *Life Transitions* (e.g., puberty or transition into high school), *Daily Hassles* (e.g., family arguments or waiting in a long line at a security checkpoint of an airport) and *Disasters* (e.g., experiencing a car accident or a computer crashing causing loss of important information). These types of environmental stressors can cause you to be fearful and have a racing heartbeat. And our perceptions of these responses can actually make the symptoms worse. It is also important to note that these stressors can be perceived differently by different people. For instance, two people can get stuck in the same elevator and while one would find the experience to be a nuisance, another will tell you it was the worse situation they have ever been in. Here is a [poll](#) about the role of stresses and stress responses in the natural world. This supplementary article explores [what is the right amount of stress](#).

PHYSIOLOGICAL VS PSYCHOLOGICAL STRESS



While most of the time we think about stress in a negative way, some stress is adaptive and can even give us an edge. Part of the stress reaction involves the secretion of hormones, which in turn will stimulate the cardiovascular system, which includes your heart. In this way, the right amount of stress may release hormones and increase our ability to focus better on an exam or to quickly maneuver our car when we are trying to avoid an accident. Most stressors in our daily life are psychological in nature—dating, exams, presentations, and deadlines, so the adrenaline and cortisol (i.e., stress

hormones) released into the bloodstream do not get burned off. These types of psychological stressors can initiate an over-activation with a tendency to make the stress response worse. So, a response to an environmental stressor may start as fear and turn into a panic attack.

Acute vs Chronic Stress



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One of the goals of our body is to maintain stability (i.e., **homeostasis**). We can, therefore, define stress as an actual or perceived threat capable of throwing our homeostasis off balance. Stress exposure starts the responses. When a person is exposed to prolonged stress, **overload** may occur. When the stress response is triggered too often and/or remains active too long, it can cause “wear and tear” on the body from lowering your immune system and bone density, to hypertension, to heart attack.

Acute stressors are observable stressful events that are time-limited such as an upcoming test or a family gathering. An acute stressor brings activation to our neuroendocrine system and makes us ready to act (i.e., “fight or flight”). Remember that pumped up feeling you got the last time you were getting ready to give a speech in front of the class? **Chronic stressors**, in contrast, are persistent demands on you; they are typically open-ended, using up your resources in coping but not having any resolution. Here is a [short article](#) and [podcast](#) on stress effects on health and suggestions for stress preventive activities.

Everyday Hassles



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[Robert Service](#), a Canadian Poet, cautioned, “Be master of your petty annoyances and conserve your energies for the big, worthwhile things. It isn’t the mountain ahead that wears you out—it’s the grain of sand in your shoe.”

In addition to many stressors in our lives being psychological and chronic in nature, we should pay attention to everyday hassles, which can be as harmful, if not more harmful than life-changing events. Everyday hassles may include things like worrying about one’s weight, having too much work with too little time, or a stressful commute to school or work. Major life changes usually bring about more hassles, which may lead to more physical stress symptoms.

In summary, stress can be adaptive—in a fearful or stress-causing situation, we can run away to save our lives, or we can concentrate better on a test. Biologists might even say it is necessary. But, stress can also be maladaptive. This is especially true if it is prolonged (i.e., chronic stress) because it increases our risk of illness and health problems. Thus, reducing stress, especially prolonged stress, is essential to healthcare. This [video](#) explains the effects of daily hassles on our health.

COPING AND STRESS



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To deal with stress in your life, it is important to figure out where that stress originates and notice how you tend to react to it. Later in this chapter, we will show you how community psychologists consider the environment and ecological perspectives as intertwined in stress and coping. Lazarus and Folkman (1984) have been among the most influential psychologists in the stress and coping field, and they defined coping as efforts to manage demands that could exceed our resources. It is important to highlight from this definition that when a person perceives a life circumstance as taxing and exceeding the resources they have, this person will experience stress. Therefore, coping involves your efforts to manage stress, which is illustrated in Figure 1.

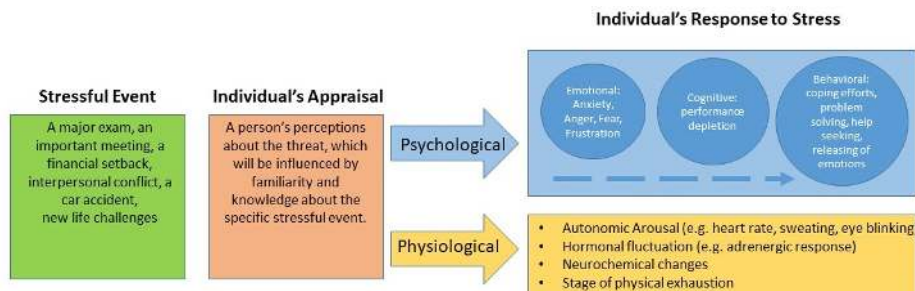


Figure 1. Overview of Stress Process and Coping Responses

Coping Defined

Lazarus and Folkman (1984) felt that when humans perceive a life circumstance as taxing and exceeding their resources, stress will be experienced, which we have already defined in the prior section as an overload of incoming information into a system. Therefore, coping involves persons' efforts to manage stress, whether the process of dealing with stress is adaptive or not (Lazarus, 1993). When we talk about coping, we will need to consider the intensity of the stressor, the context of coping, and an individual's appraisal of coping expectations.

Coping Types

Research on coping has usually found five types of **coping styles** (Clarke, 2006; Skinner, et al., 2003; Folkman & Moskowitz, 2005). These include the following: (1) **problem-focused coping style** involves addressing the problem situation by taking direct action, planning or thinking of ways to solve the problem, (2) **emotion-focused coping style** involves expressing feelings or engaging in emotional release activities such as exercising or practicing meditation, (3) **seeking-understanding coping style** refers to finding understanding of the problem and looking for a meaning of the experience, and (4) **seeking help** involves using others as a resource to solve the problem. Finally, people might respond to stressors by (5) **avoiding the problem** and trying to stay away from the problem or potential solution to the problem.

Coping Strategies

Coping strategies are the choices that a person makes in order to respond to a stressor. A strategy can be adaptive (effective) or maladaptive (ineffective or harmful). The ideal **adaptive coping** strategy varies depending on the context, as well as the personality traits of the person responding. The coping strategies can be problem-solving or active strategies, emotional expression and regulation strategies, seeking understanding strategies, help or support-seeking strategies, and problem avoidance or distraction strategies.

Shift-and-Persist (Chen & Miller, 2012), and it requires individuals to first shift views of the problem. To shift, you need to (1) recognize and accept the presence of stress, (2) engage in emotional regulation and control negative emotions, and (3) practice self-distancing from the stressor to gain an outsider's perspective of the stressful context. To persist, you would need to (1) plan for the future through goal setting, (2) recognize a broader perspective when obstacles arise, (3) determine what brings meaning to your life, and (4) become flexible to determine new pathways to goals. The [Center for Disease Control and Prevention](#) and the American Heart Association offer other coping strategies. Two more discussions on coping strategies are found in these two Ted Talk Videos [here](#) and [here](#).

Table 1 below presents a list of coping strategies and is a summary of strategies reported in Clarke, 2006; Skinner, et al., 2003; Folkman & Moskowitz, 2005. Although completed lists are more extensive, this table presents styles reported across the three studies that presented similar types of responses.

Table 1. Coping Strategies and Types of Responses

Coping Strategies:	Type of Responses:
<i>Problem Solving or Active Strategies</i>	• Work on solving the problem in the situation • Make a plan for action and follow up
<i>Emotional Expression and Emotional Regulation Strategies</i>	• Let emotion out; get in touch with feelings and let them out • Let someone know about my feelings • Keep emotions under control by performing appeasing activities • Cognitive restructuring; reorganizing the way I look at the situation
<i>Seeking Understanding Strategies</i>	• Try to understand or find meaning of the situation; looking for learning
<i>Help-seeking Strategies and Support-seeking Strategies</i>	• Seeking instrumental aid or advice from others • Seeking comfort or understanding from others
<i>Problem Avoidance Strategies and Distraction Strategies</i>	• Acting like nothing had happened • Avoid thinking or doing anything about the problem • Leaving the scenario and staying away from the stressful situation • Efforts to avoid thinking about the problem situation by using distractions or entertainment activities

To understand coping as a process, we need to understand people's reaction to stress in context. This includes assessing whether the coping thoughts or actions are good or bad for that given challenge and given context. In addition, the process of coping includes the particular person, the particular encounters with the stressor, the time of the person's reactions, and the outcome being examined. Practical Application 13.1 can help you examine your reaction to stress and understand your **coping process**.

Practical Application 13.1

Understanding Stress in Your Life

- What was stressful about it for you?
- Was it a short-term or a long-term situation?
- What thing did you do to cope with this experience?
- What resources helped you cope with this stressful experience?
- How did your experience affect you as a person?
- What did you learn or how did you grow through this experience?

COPING AND CONTEXT



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Community psychologists try to understand coping beyond individual perceptions and reactions to stress. In general, new **coping models** are looking into what the characteristics of people that will elicit a given coping style are. In addition, it is important to learn more about the effectiveness of different coping styles when used across different contexts (which includes the environment and nature of the stressors). A community psychologist is interested in understanding the contextual element that produces or reduces a given stressor, so they create interventions to help people cope with their stressors. Finding the *best fitting* coping strategy may improve people's experiences and outcomes when confronted with challenging/stressful *life transitions*.

All people are exposed to transitions, with youth being even more likely (e.g., starting elementary/middle school, finishing high school, first job, etc.), and the experience of mastery of these situations might help school-age youth successfully cope with future transitions. The following Case Study 13.1 highlights how community psychologists might use preventive strategies to help youth deal with these types of natural stressors that they will encounter in life.

Case Study 13.1

Preparing Students for High School Graduation



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Leonard Jason and Betty Burrows (1983) provided youth about to graduate from high school with practice crisis experiences to work through, so that they could generate successful outcomes for stressful situations. The youth were first informed that during times of transition, heightened levels of emotions are often experienced. These elevated levels of arousal can become maladaptive if experienced over a prolonged period of time. They then had a chance to practice techniques to effectively reduce excessive arousal or anxiety such as progressive muscle relaxation, meditation, or listening to quiet music. The youth were then given anxiety-promoting transitions to imagine. They were asked to try one of these strategies to alleviate the anxiety of this crisis situation.

The seniors also had a chance to discuss how personal beliefs and self-statements can affect one's mood. When difficult transitions are encountered, irrational beliefs or negative self-statements might arise. An example of an irrational belief is that it is a necessity for us to be loved and approved by everyone we know. The seniors were provided with situations involving transitions to role-play. Within each of these scenes, one actor adopted an irrational belief; the other attempted to show a more rational way of thinking. Following each role-play, the participants had a chance to discuss how the adoption of a rational or irrational belief might affect their ability to successfully handle transitions. Following the role-plays, all the students discussed how effective the various coping strategies might have been in resolving transitional events.

This study found that seniors about to graduate from high school can profit from being provided a preventive intervention that prepares them for dealing with transitions. Here are some of the students' comments: "You really helped me to really think out all my problems and how to answer them. Thanks!! It helped me to help other people who are in difficult situations." "I think that problem solving is the most helpful to people at my age because we face problems every day." "I feel that I have benefited from the sessions as I have gone through a lot as my mother has been divorced twice and is married for the third time and I just moved from a small town to a large city."

But sometimes, crises occur, and then we need to find ways to deal with those situations. Case Study 13.2 will provide an example of helping individuals and communities with dealing with a natural disaster, Hurricane Maria.

Case Study 13.2

Coping with Hurricane Maria



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On September 20th, 2017, Hurricane Maria devastated Puerto Rico. Watching the compounding difficulties over the course of that fall, Melissa Ponce-Rodas and a team of 20 members, four faculty, one alumna, 13 students, and two other staff members with backgrounds in undergraduate and graduate psychology, community and international development, and social work decided to spend their Spring break helping with the recovery. The ultimate goal of the mission was to "Accept, Talk, Heal," providing mental health training in churches and communities to give those affected by the disaster the tools they needed to help them through the trauma of the aftereffect of the hurricane and to help them recover beyond the physical reconstruction of their community. This [intervention](#) was about more than cleaning up debris or restoring a school, it was about empowering a community to overcome trauma.

Prior to the intervention, the team contacted local church agencies and asked about their experiences and what was truly needed. This process of inquiry resulted in another group of people with expertise in the community to be invited into the collaborative process. The local collaborators included social workers, police officers, certified nursing assistants, staff from the four schools, church groups, and others who were directly affected by the natural disaster. It is not surprising that the team was quickly accepted and trusted by the community. Ponce-Rodas believes one contributor to this is her Puerto Rican heritage that helped bridge the intervention team to the community and facilitate connections between community members and those from the mainland.

Understanding the complexity of the coping process was one of the reasons Dr. Ponce-Rodas drew on her Community Psychology experience to partner with existing agencies on the island. Because so many community members had experienced symptoms of Post-Traumatic Stress Disorder the intervention aimed at normalizing the emotional reactions affecting the individuals. The team

reminded the community members participating in the intervention that the whole community was going through this and no one should feel isolated in their experience, and that it is normal to seek help from others when faced with adversity. The team emphasized this with the “Healing is a Process” slogan, highlighting the dynamic nature of this concept.

The team also agreed that a strength-based approach fits this intervention well. To help combat the loss and desperation, the community participants were encouraged to turn to the strengths the community still had. The message across the island was “Puerto Rico Se Levanta” (Puerto Rico Will Rise). Many heroic stories of selflessness and self-sacrifices emphasized resilience in the wake of such massive destruction and highlighted the power and strength in the community. The community members also often commented on their faith, which meant that the team included faith in their intervention. The team firmly believed that the best way the community could get help was by understanding the community’s background, by recognizing the dynamic process of coping, by alleviating the stigma of help-seeking, by drawing on their own strengths, and by helping themselves.

Coping and Support



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Coping can be aided by asking others for support to help overcome problems. **Support-seeking strategies** include seeking advice or information or direct assistance. Individuals who engage in these types of help-seeking strategies are more likely to obtain social support. Seeking help from relatives may prove to be successful, which might contribute to it becoming a frequently employed coping mechanism. Barker (2007) suggests that youth’s help-seeking behaviors set up the conditions to create a rich supportive network for them, and the feeling that there is available support. Case Study 13.3 illustrates how one can go about studying supports using multiple points of view.

Case Study 13.3

Support Seeking Behavior



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Having at least one friend in a recovery home was found to be about the best predictor among residents of recovery homes of having a good outcome, which involves not using drugs or engaging in illegal activities. To better understand what seeking support might be about, several community psychologists used focus groups to better understand natural friendship and mentoring relationships. For example, participants were asked about how they determined who they would go to in the house for support, the type of support they received from housemates, and the characteristics that those individuals have. The findings from the focus groups had a theme of promoting social support within their recovery homes, as one female participant explained:

“When I come in this door and I’ve got something to talk about, I don’t care which of these girls is here, I’m just going to talk about it. And that’s because they play a positive mentor or role model in my life. I don’t have to pick out two or three or one to say who I want to talk to.”

Instrumental supports were often related to the ability of other house members to provide tangible support for residents. As one female participant described:

“I have not been employed . . . I’ve always been concerned about if my rent’s going to get paid on time. These sisters came together and told me if you can’t get your rent paid, we know what you’re doing and we know that you want to be here and they were willing to go into their own pockets to help me pay my rent.”

Focus-group themes indicated that men were also able to form supportive relationships within the recovery home settings, but not as quickly as the females (Lawlor et al., 2014).

RESILIENCE



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Individuals who experience significant and chronic stressors are often referred to as being “**at-risk**” of something, whether it be poor school performance, problems with alcohol or drugs, or engaging in illegal activities. However, not all individuals “at risk” of negative outcomes end up struggling with the outcomes. Some people are able to avoid negative outcomes and even thrive despite the adversity they face. Why is it that some people are successful in spite of seemingly insurmountable obstacles?

This question was at the heart of early studies of resilience. **Resilience** is a dynamic process characterized by positive outcomes despite adversity or stress (Luthar, Crossman, & Small, 2015). In other words, resilience refers to how people maintain, or in some cases develop, healthy and positive outcomes in spite of stressful situations. The study of resilience stemmed from researchers who began to notice that a subset of their participants, often children facing significant adversity, did well despite their difficult circumstances. For example, Garmezy (1974) studied children of parents with schizophrenia. Among this group of at-risk children, all were expected to struggle in various aspects of life and likely develop schizophrenia. But a subset of children exhibited surprisingly positive and adaptive behavioral patterns despite their level of risk. Another large-scale study recruited all of the children born on the island of Kauai, Hawaii (Werner, 1996). The original goal of the study was to assess the long-term consequences of stressful living environments (e.g., family discord, divorce, parental alcoholism, mental illness). Most of the children living in these stressful

environments struggled academically and behaviorally. However, one-third of these “high-risk” children did not develop learning or behavioral problems; in fact, many of them thrived. Studies like these helped to shift our focus from a deficits-only approach to one more able to consider both deficits and strengths.

“Resilience does not come from the rare and special qualities, but from the everyday magic or ordinary, normative human resources in ... children, in their families and relationships, and in their communities.” (Masten, 2001, pp. 235)

Resilient children were thought to have been invulnerable and able to weather any storm. Traits found to characterize resilience include high creativity, effectiveness, competence, and ability to relate well to others. Now, resilience is viewed as the interaction between the person and their environment, and given the right combination of individual and environmental supports, it might be possible for anyone to be resilient. From a Community Psychology perspective, research had found that these children are positively affected by their immediate and extended family networks and religious organizations (Wright et al., 2013).

So far, we have considered resilience as an individual construct. Individuals can be resilient to adversity. However, it is also possible to apply this idea of resilience to groups of people. **Community resilience** is the collective ability of a defined group of people to deal with change or adversity effectively (Aldrich & Meyer, 2015). When adversity, like a disaster, financial struggle, or war strikes a community, will the community as a whole be able to overcome and bounce back?

Resilient communities often have many characteristics in common. Communities that are resilient frequently have access to both resources and relationships that support resilient outcomes. An important element of community resilience includes members’ knowledge of their own community, both its weaknesses and strengths. In addition, resilient communities have strong community social networks in which people work together to achieve goals, with competent governance and leadership. Often there is also an economic investment, both before and after adversity strikes. Another important factor is individual, family, and government preparedness. And finally, resilient communities have positive attitudes and an acceptance of change (Patel et al., 2017). Both research and community work is now being done to help communities build these resources and relationships to protect against adversity.

Case Study 13.4

Promoting Community Resilience



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From 2000 to 2010, poverty rates in the Belmont Cragin neighborhood in Chicago doubled. Consisting of 80% Latino residents, Belmont Cragin residents have experienced soaring poverty rates associated with the gentrification of nearby neighborhoods. In addition, many residents in the neighborhood experienced significant trauma when they were younger. These adverse childhood experiences include living in extreme poverty; suffering physical, sexual, or emotional abuse; being exposed to community violence; having a parent struggling with substance abuse, or many other potentially traumatic experiences. Having adverse childhood experiences can be detrimental to emotional and physical health, and individuals who have experienced them are more likely to experience additional negative emotional or behavioral outcomes. In recognition of the increasing poverty in the neighborhood and the trauma experienced by many residents, community leaders formed the Resilient [Belmont Cragin Community Collaborative](#). Community Psychologists Suzette Fromm Reed and Judith Kent worked with Belmont Cragin leaders to help residents cope with adverse childhood experiences by facilitating trauma-informed programming at schools using mentoring, tutoring, and counseling to help at-risk youth stay on track. They also helped train police to de-escalate the conflict. The Resilient Belmont Cragin Community Collaborative utilized existing community resources and established partnerships with resources outside of the community to ensure collective healing and growth. It brought together members of the community, from schools, health care settings, businesses, police departments, families, faith leaders, and others, to help residents address these traumatic experiences and thrive. Programs like these exemplify community resilience and help individuals, and the community as a whole, grow and heal together.

There is a need to apply this type of work to help those from disadvantaged neighborhoods and communities who frequently experience chronic stressors. This might be done by promoting the use of multilevel and interdisciplinary work that meshes with Community Psychology’s values of promoting social justice, as indicated in Chapter 1 (Jason et al., 2019).

SUMMING UP



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We all experience stress. However, we respond to this stress in different ways. Sometimes low levels of stress can actually be helpful as it could motivate you to study for an exam. Although the experience of stress is very subjective, stress elicits physiological, emotional, and cognitive reactions in us all. To deal with these stressors, we mobilize resources for coping with the problems confronting us. The success of our coping efforts will depend on ourselves as well as the environmental challenge. For example, most of us have the resources to deal with the stress of a thunderstorm, but we might really be challenged if we are confronted with a tornado that comes through our neighborhood. So there are different levels of stressors that we face. In this chapter, we examined the relationship between stressors and coping, and we reviewed the different coping styles and the relationship between individual and context and coping outcomes, including resilience. We hope that this review of stress has provided you with some new insights about how you might use a variety of coping strategies to deal with stress and to work toward the reduction of stress among others.

Critical Thought Questions

1. What are the most difficult stressors for you?
2. With these most difficult stressors, what are the best ways you have found to cope with them?
3. Are there people you go to when you are having problems at college? What are the ways they are helpful and what are the ways they might change to be even more helpful to you?

Take the [Chapter 13 Quiz](#)

View the [Chapter 13 Lecture Slides](#)

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CHAPTER OVERVIEW

3: Chapter 3

[3.1: Ethics](#)

[3.2: The Development of Dual and Multiple Relationships for Social Workers in Rural Communities](#)

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3.1: Ethics

Skills to Develop

- Discuss how research involving human subjects is regulated
- Summarize the processes of informed consent and debriefing

Today, scientists agree that good research is ethical in nature and is guided by a basic respect for human dignity and safety. However, as you will read in the feature box, this has not always been the case. Modern researchers must demonstrate that the research they perform is ethically sound. This section presents how ethical considerations affect the design and implementation of research conducted today.

Research Involving Human Participants

Any experiment involving the participation of human subjects is governed by extensive, strict guidelines designed to ensure that the experiment does not result in harm. Any research institution that receives federal support for research involving human participants must have access to an **institutional review board (IRB)**. The IRB is a committee of individuals often made up of members of the institution's administration, scientists, and community members. The purpose of the IRB is to review proposals for research that involves human participants. The IRB reviews these proposals with the principles mentioned above in mind, and generally, approval from the IRB is required in order for the experiment to proceed.



Figure 3.1.1: An institution's IRB meets regularly to review experimental proposals that involve human participants. (credit: modification of work by Lowndes Area Knowledge Exchange (LAKE)/Flickr)

An institution's IRB requires several components in any experiment it approves. For one, each participant must sign an informed consent form before they can participate in the experiment. An **informed consent** form provides a written description of what participants can expect during the experiment, including potential risks and implications of the research. It also lets participants know that their involvement is completely voluntary and can be discontinued without penalty at any time. Furthermore, the informed consent guarantees that any data collected in the experiment will remain completely confidential. In cases where research participants are under the age of 18, the parents or legal guardians are required to sign the informed consent form.

While the informed consent form should be as honest as possible in describing exactly what participants will be doing, sometimes deception is necessary to prevent participants' knowledge of the exact research question from affecting the results of the study. **Deception** involves purposely misleading experiment participants in order to maintain the integrity of the experiment, but not to the point where the deception could be considered harmful. For example, if we are interested in how our opinion of someone is affected by their attire, we might use deception in describing the experiment to prevent that knowledge from affecting participants' responses. In cases where deception is involved, participants must receive a full **debriefing** upon conclusion of the study—complete, honest information about the purpose of the experiment, how the data collected will be used, the reasons why deception was necessary, and information about how to obtain additional information about the study.

DIG DEEPER: Ethics and the Tuskegee Syphilis Study

Unfortunately, the ethical guidelines that exist for research today were not always applied in the past. In 1932, poor, rural, black, male sharecroppers from Tuskegee, Alabama, were recruited to participate in an experiment conducted by the U.S. Public Health Service, with the aim of studying syphilis in black men. In exchange for free medical care, meals, and burial insurance, 600 men agreed to participate in the study. A little more than half of the men tested positive for syphilis, and they served as the

experimental group (given that the researchers could not randomly assign participants to groups, this represents a quasi-experiment). The remaining syphilis-free individuals served as the control group. However, those individuals that tested positive for syphilis were never informed that they had the disease.

While there was no treatment for syphilis when the study began, by 1947 penicillin was recognized as an effective treatment for the disease. Despite this, no penicillin was administered to the participants in this study, and the participants were not allowed to seek treatment at any other facilities if they continued in the study. Over the course of 40 years, many of the participants unknowingly spread syphilis to their wives (and subsequently their children born from their wives) and eventually died because they never received treatment for the disease. This study was discontinued in 1972 when the experiment was discovered by the national press (Tuskegee University, n.d.). The resulting outrage over the experiment led directly to the National Research Act of 1974 and the strict ethical guidelines for research on humans described in this chapter. Why is this study unethical? How were the men who participated and their families harmed as a function of this research?



Figure 3.1.2: A participant in the Tuskegee Syphilis Study receives an injection.

For more information on this and other studies that contributed to our current thoughts on ethics in research, watch this video:





Summary

Ethics in research is an evolving field, and some practices that were accepted or tolerated in the past would be considered unethical today. Researchers are expected to adhere to basic ethical guidelines when conducting experiments that involve human participants. Any experiment involving human participants must be approved by an IRB. Participation in experiments is voluntary and requires informed consent of the participants. If any deception is involved in the experiment, each participant must be fully debriefed upon the conclusion of the study.

Glossary

debriefing

when an experiment involved deception, participants are told complete and truthful information about the experiment at its conclusion

deception

purposely misleading experiment participants in order to maintain the integrity of the experiment

informed consent

process of informing a research participant about what to expect during an experiment, any risks involved, and the implications of the research, and then obtaining the person's consent to participate

Institutional Review Board (IRB)

committee of administrators, scientists, and community members that reviews proposals for research involving human participants

Contributors

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3.2: The Development of Dual and Multiple Relationships for Social Workers in Rural Communities

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CHAPTER OVERVIEW

4: Chapter 4

[4.1: Human Rights](#)

[4.2: The Sociocultural Model and Therapy Utilization](#)

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4.1: Human Rights

Learning Objectives

- Learn from the national and global perspectives on human rights for immigrants and refugees.
- Recognizing that the world is constantly and rapidly changing.
- Recognizing that Global/national/international events can have an impact on individuals, families, groups, organizations, and communities.
- Global implications dictate that we foster international relationships and opportunities to address international concerns, needs, problems, and actions to improve the well-being of not only U.S. citizens, but global citizens.

Introduction

Fatima and her three girls, ages 18 months, 4 and 8 years old, were exhausted by the time they reached the border between the United States and Canada. They had been on the bus from New York for two whole days: it had been very hard changing the baby's diapers in the tiny bus bathroom that was their only option. Though Fatima had proper passports from the African nation they had fled, she told immigration officials that she was actually fleeing a neighboring country, since there was civil war raging and she thought they would be more likely to let her and her family into Canada. But since the passports did not match her story, United States officials confiscated all of their identity documents and sent her and the girls back to New York. Penniless and alone, the little family made their way to Chicago, where Fatima had heard they were more generous to refugees like herself. What was she fleeing with her three little girls? The prospect of their having to undergo the same female genital mutilation procedure that she had endured at the age of 9. Though their future was extremely unsure, Fatima knew that she had done the only thing she could to protect her girls. "In my country, they circumcise the boys in the hospital under anesthesia," she told her therapist at the free clinic. "But the girls – no – the girls are circumcised in the bush with rusty razors and no anesthetic at all. This is what the girls get." For Fatima, there was no other choice for her but to flee and take the girls with her. Even her husband agreed though he had stayed behind in their home country to try to keep sending them money and support. And so, Fatima was alone with her children in a foreign land, hoping for help and guidance with navigating the new language, customs, culture, and realities of the United States.

Background

There are currently an estimated 263,000 refugees and 84,300 asylum seekers residing in the United States (UNHCR; United Nations High Commissioner for Refugees, 2013). These estimates are staggering but will continue to increase in the subsequent years as a result of ongoing armed conflict and political unrest around the world. If it were not for the significant and ongoing international human rights violations, the number of displaced families – both immigrants and refugees – would be considerably smaller. Families flee their home countries for a range of reasons; from escaping oppressive regimes, as is the case in Syria and Iraq, to attempting to better their economic situations, as is the case in Mexico and much of Latin America. The United States, amongst other Western countries, regularly sees influxes of immigrant and refugee families from around the world depending on the sociopolitical and historical context of the time. Some of these families are intact, but the vast majority are scattered and separated around the world.

According to the Migration Policy Institute (MPI, 2019), the top five countries of origin for resettlement in the United States are the Democratic Republic of Congo, Burma, Ukraine, Eritrea, and Afghanistan. President Trump's announcement of additional screening for refugees from certain countries may shift these demographics somewhat (MPI, 2015). The controversy over admitting refugees into the United States from certain parts of the world, including Syria, continues to rage unabated in the mainstream press and online. For example, the fear following the Paris attacks of November 2015 prompted governors of 31 states to refuse to admit Syrian refugees; although they do not have the power to control nationality laws (Barajas & Frazee, 2015).

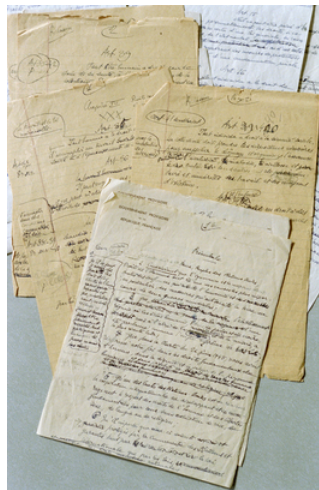
The purpose of this chapter is to provide a broad overview of human rights and lay out some of the essential concepts that are critical to understanding how this global issue affects immigrant and refugee families. We will present the history and general theories of human rights law, as well as explore how various issues pertaining to the current relationship between international human rights law and domestic sovereignty are being dealt with in the United States. Additionally, we will examine how specific human rights issues impact refugee and immigrant families in the United States. Implications for research, policy, and practice, questions for further discussion, and a case study can be found at the end of the chapter.

Kirsten Lind Seal (Owner-operator at Lind Seal Counseling & Consultation LLC) and Damir S. Utržan (Family Social Science, University of Minnesota)

What are Human Rights?

António Guterres, the United Nations High Commissioner for Human Rights (UNHCR), wrote that the “...UNHCR has never had to address so much human misery in its 64-year history” (Project Syndicate, 2015). This maxim guides the importance of understanding and exploring the intricacies of human rights. The foundational definition of *human rights*, according to the UNHCR, encompasses “...inherent rights to all human beings, whatever the nationality, place of residence, sex, national or ethnic origin, color, religion, language, or any other status.” This definition is critical to understanding how and why the international community endeavors to define and protect these rights. A complete understanding of human rights includes moral values, ethical and philosophical norms such as autonomy, justice, beneficence, and non-maleficence. All of these characteristics factor into the creation of a particular paradigm of rights for human beings that has been specifically and gradually shaped into international law since the United Nations was founded in 1945.

While human rights are based on moral values, it is important to recognize that values are fundamentally different from rights. The concept of values addresses *what is important* whereas human rights address social practices that seek to empower human beings. A right is not merely a benefit since having a right also gives a person a significant legitimacy within the system of governmental authority. According to Donnelly (2003), “...a human rights conception of human dignity and political legitimacy rests on the fact that human beings have an essential, irreducible moral worth and dignity irrespective of the social groups to which they belong” (p. 27). This means that universal human rights, for the purpose of this discussion, are rights that have been codified (i.e., incorporated into a legal code) by the international community.



Early version of the Universal Declaration of Human Rights. United Nations Photo by Greg Kinch – public domain.

While human rights are based on an ideal, they also provide a social means of ensuring that nation-states grant to all human beings the opportunity to lead a life of human dignity; a life worthy for a human being. The specifics of exactly what human rights should entail have been enshrined in documents such as the Universal Declaration of Human Rights (UDHR, 1993), which conceptualizes the idea of human rights as a “self-fulfilling moral prophecy” (Donnelly, 2003, p 15). The UDHR has led to several international treaties, such as the International Covenant on Civil and Political Rights and the International Covenant on Economic Social, and Cultural Rights. These statutes and treaties are created by societies coming together in order to codify global requirements for the kind of human behavior that will support the thriving of all. Thus the notion of human rights as a social practice is integral to the process of understanding the impact on immigrant and refugee families.

The Universal Declaration of Human Rights

In response to the gross human rights violations following the Second World War, the UDHR was chartered by the United Nations in an attempt to prevent such atrocities from being committed again. The UDHR’s preamble states unequivocally “recognition of the inherent dignity and of the equal and inalienable rights of all members of the human family is the foundation of freedom, justice and peace in the world” (OHCHR; Office of the High Commissioner for Human Rights, 1948). Following the preamble are 30 Articles, which lay out in detail the specific rights to which all human beings should be entitled. According to the UNHCR, there are 389 different translations of this document.



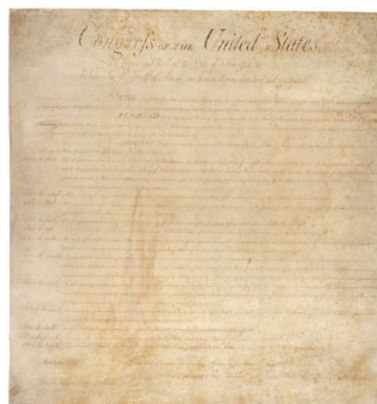
Poster depicting the Universal Declaration of Human Rights, English Version. United Nations Photo – public domain.

The UDHR encompasses both *negative* (i.e., the right to not be tortured, imprisoned without cause, or enslaved) as well as *positive* (i.e., the right to own property, the right to freedom of thought, and the right to marry) rights. It also enumerates, for the first time, the core principles of human rights, which are: universality, inter-dependence and indivisibility, equality, and non-discrimination. The UDHR states further that human rights are not merely an entitlement, they also include rights and obligations. In other words, having a right brings with it a particular obligation as well. If we are to enjoy our rights as humans, then we need to respect these same rights for others. These concepts have been reiterated in many subsequent international human rights treaties, declarations, and conventions. This document (UDHR), along with the International Covenant on Civil and Political Rights (ICCPR) and the International Covenant on Economic, Social, and Cultural Rights (ICESCR), are considered to be the foundational pillars of international human rights law and the legal basis for all subsequent human right norms, standards, and rules.

The Status of Human Rights in the United States

While human rights have in large part been internationalized, they have to be implemented at the domestic level. According to Donnelly (2003), this dichotomy permits countries to fulfill dual and seemingly incompatible roles: essential protector and principle violator. In the United States, this duality can be seen in the difference between the laws upon which the country was founded and the implementation of these laws in an equitable fashion.

The Bill of Rights, as codified in the United States Constitution, lays out specific human rights that parallel those to which the majority of international human rights regimes adhere. Thus, the founding myths of this country are grounded in the central place of human rights (Donnelly, 2003). In fact, many if not most liberal democracies share these constitutive principles. As Koopmans (2012) points out, “internal constitutive principles – such as the right to exercise one’s religion...imply that the granting of rights to individuals and groups will be more similar across democracies than it will be between them and non-democracies” (p 25). And yet, there remain significant areas where United States domestic policy can be seen to violate various rights of various portions of the population at any given time.



Political Issues

The most pressing human rights issues in the United States revolve around immigrant and refugee families. The strategic priorities outlined by the UNHCR include: (a) countering discrimination; (b) combating impunity and strengthening accountability; (c) pursuing economic, social and cultural rights and combating poverty; (d) protecting human rights in the context of migration; (e) protecting human rights during armed conflict, violence and insecurity; and (f) strengthening international human rights mechanisms and the progressive development of international human rights law. Priorities (a), (c) and (d) make up the elements most germane to the human rights situation in the United States today. The difficulties faced by immigrant and refugee families include classism, racism, sexism, and discrimination on the basis of religion as well as uncertain economic circumstances.



Secretary-General of the United Nations, Ban Ki-moon, meets President of the United States, Barack Obama. UN.org – public domain.

The United States voted in favor of the UDHR but it did not ratify (i.e., sign) the document. While various theories attempt to explain relevant reasons, numerous rights enshrined in the UDHR are in the Constitution and Bill of Rights (AHR; Advocates for Human Rights, n.d.) The United States' apparent sense of exceptionalism to international standards and norms has been evidenced over time in two main ways: the ongoing torture of Guantanamo Bay detainees and the revelation that American social scientists were involved in reverse engineering torture techniques for the government. While the United States may at times act outside of the limitations established by the international community (and specifically the UDHR) this stance is not the focus of this chapter. As the UNCHR notes, "national and local politicians have sought to mobilize electoral support by promoting xenophobic sentiments, exaggerating the negative impact of hosting refugees while ignoring the fact that refugees can actually attract international assistance and investment to an area, creating new jobs and trading opportunities" (2006, p 32). In this way, the refugee situation has often been used as a political football in United States political culture.

Video

Ruben Parra-Cardona, Ph.D., LMFT discusses discrimination and systemic issues affecting Latino immigrants in the United States (6:30-9:19).



Legal Issues

The current legal climate in the United States is negatively skewed against international human rights, particularly as it pertains to the legal status of displaced persons (persons who are forced to leave their home country due to war, persecution or natural disasters). There are many reasons to be pessimistic about successfully using international human rights arguments as a way of advancing displaced person's rights in the United States (Chilton, 2014; Cole, 2006; ICHR; International Council on Human Rights, 2008). According to Cole (2006), in spite of its history as a nation of immigrants, the United States remains deeply nationalist and quite parochial; the law reflects that parochialism. Furthermore, "International human rights arguments are often seen as the advocates' last refuge pulled out only when there is no other authority to cite" (Cole, 2006, p. 628).



Poster entitled, "It is our right to seek and enjoy in other countries asylum from persecution." UN Photo – public domain.

However, this trend seems to be moving the nation towards the transnational in terms of how human rights law is perceived and implemented in the legal system and culture of the United States. This means that increased globalization and interdependence has had the effect of strengthening the influence of international human rights standards in the United States. The hope is that these standards may "command greater respect from our own domestic institutions" (Cole, 2006, p 643). Cole further posits that the paradigm shift in the United States from national to transnational, merging the national and the international, parallels the shift in the United States from state to federal power that occurred with the advent of the New Deal in the 1930s. In other words, there is reason to hope that gradual change is coming within the legal system in the United States with regards to its acceptance of the international human rights regimes, norms, and standards.

Refugee families and asylum seekers. The terms of refugee and asylum seeker are often used interchangeably, but there are important legal differences between them. These differences not only determine which resources they are eligible for once arriving in the United States but also in which phase of the legal process they are currently.



Refugees and displaced persons in South-East Asia; Cambodia, Vietnam, and Laos. United Nations Photo – Coping with Disaster – CC BY-NC-ND 2.0.

Refugees. An estimated 51.2 million people were displaced since 2013 as a direct result of persecution, war, violence, and human rights atrocities (UNHCR, 2018). In 2017, USCIS received 139,801 affirmative asylum applications and the EOIR received 119,303 defensive asylum applications, but only 26,568 applications were approved (DHS, 2019). The remaining applications were abandoned (1,439), withdrawn (6,400), or simply unaccounted for (11,391). Being that the recent United States population estimate is 318 million people, refugees make up less than 1% of the population. The families seeking asylum from their home countries often have significant traumatic histories and thus can loom larger in the public sphere than other types of immigrants. Most of these families are fleeing extreme injustices in their home country, such as war, political instability, genocide, and severe oppression. Because of the uncertainty of their original situation, it remains quite difficult for the Department of Homeland Security (DHS) to determine who is legitimately eligible for asylum.

Asylum seekers. A further complication for government agencies lies in trying to determine when and how to return rejected asylum seekers to their home countries (Koser 2007). Within the domain of international migration studies, there has been traditionally a differentiation made between refugees (involuntary migration) and labor seekers (voluntary migration). While the former group represents the political outcome of global systems and interactions and the latter group represents the economic outcome, nonetheless, it is quite clear that people migrate for a whole complex series of reasons, including social ones (Koser, 2007). If an asylum seeker's claim for asylum is denied, they are placed in deportation proceedings. During this process, an immigration judge (IJ) works with the asylum-seekers' attorney to determine the removal process. It is important to note that displaced persons are rarely detained and/or immediately placed on the next flight to their country of origin.

Women and Children's Rights

The UNHCR has, within the last decade, specifically recognized gender as a fundamental human rights issue. The policy on refugee women is based on the recognition that becoming a refugee affects men and women differently: "... even where there is no armed conflict, women and children continue to be subject to serious human rights violations resulting from discrimination and/or violence against them due to their gender..." (Zeiss Stange, Oyster, & Sloan, 2012). Many of the human rights issues that involve women and children obviously impact families in a very deep way. This category of violation stems from historical perceptions of women and children as property or "chattel."



International Women's Day March for Gender Equality and Women's Rights. UN Photo – public domain.

Domestic violence. The issue of domestic violence, for example, is one that disproportionately affects women and children. In immigrant families from more patriarchal societies, the home is still considered the woman's domain whereas earning is considered the man's, even when both work for pay outside the home. This particular division of labor can increase the power imbalance in these relationships, which can create a setting within which domestic violence may be more likely to occur (Perilla, 1999). This imbalance can become particularly problematic when the power hierarchy between parents and children is inverted once they arrive in the United States. Since wage-earning immigrant women often gain autonomy and greater gender equity, while men tend to lose ground, this adds further threats to male self-esteem that is already being eroded by classism, racism, and legal status (Mahler & Pessar, 2006).

Children's issues. Children are disproportionately impacted in other ways as well. Children of displaced families, asylum-seekers in particular, are more likely to be without health insurance and have less access to public programs across nationalities (Blewett, Johnson, & Mach, 2010). This is due to their legal designation of being persons who are ineligible for public services (i.e., health insurance). The literature suggests that traumatic events impact each family member regardless of whether they were directly or indirectly exposed. This is important because traumatic stress, loss, and grief extend beyond individual family members and influence the entire family (Nickerson, Bryant, Brooks, Silove, Steel, & Chen, 2011). Detention and deportation, in particular, pull families apart and make it much more difficult for parents and other caregivers to access necessary resources for their children. This is true for many immigrant families because there are many different legal statuses within families due to, for example, children being born to undocumented parents in the United States. Thus the tendency is for these families to be more careful and anxious about seeking out services that they might qualify for because of the fear of being reported to immigration authorities.

Female genital mutilation. Female genital mutilation (FGM), also known as female genital cutting, is a human rights issue of growing importance due to the increasing number of refugees arriving in the United States from East and West Africa. This practice, sometimes also known as female circumcision, is a long-standing cultural tradition in some communities. Although FGM is generally practiced in Muslim communities, there is no actual religious mandate for it (Cook, Dickens, & Fathalla, 2002). While the practice is deeply cultural, it is illegal in many African nations. However, regardless of legality, the practice is widespread. Fatima, for example, from the opening story, told the first author, "The president's daughter has been circumcised – how will he enforce this law? Are they going to put him in jail? Hah!" (Personal communication, 2011).



Angélique Kidio and Concer Sponsors Brief on Efforts to End FGM. UN Photo – public domain.

According to Mather and Feldman-Jacobs (2015), over 500,000 girls and women have undergone genital mutilation in the United States. FGM is regarded as a human rights issue for women because it can cause severe health sequelae. Even though the family of the child may consent to the procedure, this does not make it a legal practice as consenting to physical mutilation can never be legal (Cook, et al., 2002). However, recommendations include more education and counseling of women as opposed to the firmer application of the law in cases like these. This practice remains illegal in the United States, which may have the effect of pushing those refugees wishing to practice this even further underground. In other words, shame and stigma often accompany FGM. And though it can be presented as a convincing claim for asylum in the United States, many women do not feel comfortable doing so and instead pursue other means (USCIS; United States Citizenship and Immigration Services, 2019).

In 2012, the United Nations issued an interagency resolution calling for the elimination of FGM worldwide. The resolution states unequivocally that “seen from a human rights perspective, the practice reflects deep-rooted inequality between the sexes, and constitutes an extreme form of discrimination against women” (WHO; World Health Organization, n.d.). The hope is that this ban will speed the process of eliminating this dangerous and painful practice worldwide.

Video

This video discusses the border crisis and children separated from their families (0:00-7:06).



Sex Trafficking and Human Trafficking

The United Nations *Protocol to Prevent, Suppress, and Punish Trafficking in Persons, Especially Women and Children*, defines *trafficking* as the “...recruitment, transfer, harboring or receipt of persons, by any means of threat or force...for the purpose of exploitation.” This crime is globally categorized as either sex trafficking or labor trafficking. According to the DOJ (2006), there have been an estimated 100,000 to 150,000 sex trafficking victims in the United States since 2001. Furthermore, estimates of persons currently in situations of forced labor or sexual servitude in the United States range from 40,000 to 50,000.

The leading countries of origin for foreign victims in fiscal year (FY) 2011 were Mexico, Philippines, Thailand, Guatemala, Honduras, and India (DOJ, 2012). In 2011, “notable prosecutions included those of sex and labor traffickers who used threats of deportation, violence, and sexual abuse to compel young, undocumented Central American women and girls into hostess jobs and forced prostitution in bars and nightclubs on Long Island, New York” (DOS, 2012). According to the International Labor Organization (ILO, 2019), globally an estimated 4.5 million women, men, and children are sexually exploited. While there is some legal benefit (a self-petitioned visa in the United States) in place for those who cooperate in prosecuting their traffickers; with this visa, victims can receive four years of legal status. Unfortunately, far fewer receive immigration aid than are identified as victims of sex trafficking, (DOS, 2012).

Human trafficking is another area where issues of physical safety and sexual exploitation of immigrant and refugee women and children come to the forefront as a human rights issue. Contrary to popular thought, sex trafficking is an ongoing and insidious activity that also includes young boys, and the prevalence of human and sex trafficking in the United States disproportionately affects the more vulnerable, under-resourced populations such as immigrant and refugee families (DOS, 2012).

Mixed Status (Deportation) and Separation of Families

One of the most pressing human rights issues for displaced persons in the United States today is the mixed-status families (i.e., documented and undocumented). These are families whose members hold different levels of legal status in the country. Some members of the family may be documented persons (such as asylum-seeker, permanent resident or citizen) while others have undocumented status. Though the children born to undocumented migrants typically receive citizenship by birth, this does not change their parents’ legal status. The exception, however, is when undocumented parents return to their country of origin and wait until that child is 18 years of age; at that point, the young adult child can sponsor them in becoming United States citizens. When families consist of members whose legal status is documented as well as undocumented, this uncertain distal context can set the stage for significant vulnerabilities within the family.

Brabeck and Xu (2010), who studied of the effects of detention and deportation on children of Latino/a immigrants, found that the legal vulnerability of Latino/a parents, as measured by immigration status and detention and deportation experience, predicted child

well-being. In other words, the children suffer when they cannot be sure whether their parents will be able to stay and live with them in the United States on a day-to-day basis. Kanstroom (2010) writes that although “international law recognizes the power of the state to deport noncitizens, international human rights law has also long recognized the importance of procedural regularity, family unity, and proportionality. When such norms are violated the State may well be obligated to provide a remedy” (p. 222). Once again the paradox of international human rights norms conflicting with the actual social and political practices of the United States; as of this writing, the issue remains a political football in the United States.

Detention Without Trial

In 2011, the United States Congress passed the National Defense Authorization Act (NDAA) that codified, for the first time since the McCarthy era, indefinite detention without charge or trial. Subjecting refugees to detention induces unnecessary psychological fear and harm. Furthermore, it does not uphold the fundamental human rights principles set out in the ICCPR preamble (Prasow, 2012). The notion that people, whether citizens, documented or undocumented immigrants, could be held by the government indefinitely without access to the protections enshrined in the United States Constitution is a clear violation of international human rights law and anathema to human rights and civil liberties groups. As of late 2012, members of Congress proposed to have it repealed or amended. As noted by Senator Dianne Feinstein of California, “Just think of it. If someone is of the wrong race and they are in a place where there is a terrorist attack, they could be picked up, they could be held without charge or trial for month after month, year after year. That is wrong” (Prasow, 2012). The amendment that Senator Feinstein proposed, however, would protect only citizens and lawful residents; undocumented immigrants would still be subject to this odious practice.

Emerging Directions

While there is a lack of human rights literature that specifically deals with or involves the family unit, the United Nations recognizes that “...family is the basic unit of society” (UN, n.d.). As such, there is an undeniable connection between the status of immigrant and refugee families and how the United States deals with their human rights in a variety of ways. Issues such as FGM, the deportation of undocumented immigrants that splits the family between those with and without citizenship or documents, longtime detention of family members, sex trafficking, and other pressing human rights issues all have significant deleterious effects on families in this country. While much of the conversation revolves around rights and obligations for the individual as well as for the community, there is very little in the way of specific family references in the human rights literature.

Given that the UDHR focuses on individual and state actors, it is understandable that there is this gap in the research regarding how human rights issues specifically affect families. However, there needs to be a significantly deeper understanding of these issues if we are to be able to truly support immigrant and refugee families to thrive and flourish in the United States. Some questions that need to be answered are: *How do families have a unique lens on their situations? Does the family structure provide a protective factor for its members? How do women’s and children’s issues play out in this arena?* Perhaps the most pressing need for further research concerns the issue of how mixed-status immigrant families cope with the uncertainty regarding living with different levels of documentation and legal status within the same family.

Video

True Thao, MSW, LICSW discusses his perspectives on human rights and basic values such as respect and equality (1:45-3:44).



End-of-Chapter Summary

Case Study

Anna, a bright and extroverted 26-year-old from a Central American country, has just arrived at a counseling center presenting with severe depression and anxiety. Several years ago, three members of one of the most brutal guerrilla regimes in her home country held her hostage at gunpoint and sexually assaulted her for several hours. Her apparent crime was that some months before she had led a march for women's rights at the college she attended. Members of the guerilla group broke up the march and then beat the young women and men, many of whom need hospitalization. Anna was among them and was hospitalized for five days of treatment. Several months later, three members of this group surprised Anna at home; they terrorized, raped, and threatened her numerous times with death before eventually leaving with further threats if she dared to protest publicly again for women.

After this last incident, Anna fled her home country and came to the United States through a circuitous route. She had no option but to use smugglers for much of the journey. She had had to leave so abruptly and had so few resources that she left her three-year-old son behind with her grandmother. Grandmother sends Anna pictures of her little boy regularly via text message, but Anna is devastated every time she thinks of him. In order to get through her day, she tries to put him out of her mind. It is clear that this effort and the loss that she feels for her son is serious. She is currently seeking asylum in this country and, because she needs to support herself though she has no documents yet, is working as a nightclub dancer. She does not feel hopeful about her asylum application because she is worried that no one will believe her story. Furthermore, she despairs over ever being able to bring her young son to the United States since she fears that they will jail him at the border.

Discussion Questions

1. What sort of information do you need that would help you understand Anna's case better?
2. How might Anna be at continued risk for human rights violations?
3. What exposures to trauma-inducing experiences are affecting Anna? How might professionals working with immigrant and refugee populations emphasize positive adaptive skills/resilience that focus on individual and family strengths?
4. What other community resources might be helpful in Anna's situation?
5. Discuss the importance of coordinating and integrating different community services for supporting immigrant and refugee resettlement.

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4.2: The Sociocultural Model and Therapy Utilization

Learning Objectives

- Explain how the sociocultural model is used in therapy
- Discuss barriers to mental health services among ethnic minorities

The sociocultural perspective looks at you, your behaviors, and your symptoms in the context of your **culture** and background. For example, José is an 18-year-old Hispanic male from a traditional family. José comes to treatment because of depression. During the intake session, he reveals that he is gay and is nervous about telling his family. He also discloses that he is concerned because his religious background has taught him that homosexuality is wrong. How does his religious and cultural background affect him? How might his cultural background affect how his family reacts if José were to tell them he is gay?

As our society becomes increasingly multiethnic and multiracial, mental health professionals must develop **cultural competence** (See figure 4.2.1), which means they must understand and address issues of race, culture, and ethnicity. They must also develop strategies to effectively address the needs of various populations for which Eurocentric therapies have limited application (Sue, 2004). For example, a counselor whose treatment focuses on individual decision making may be ineffective at helping a Chinese client with a collectivist approach to problem solving (Sue, 2004).

Multicultural counseling and therapy aims to offer both a helping role and process that uses modalities and defines goals consistent with the life experiences and cultural values of clients. It strives to recognize client identities to include individual, group, and universal dimensions, advocate the use of universal and culture-specific strategies and roles in the healing process, and balances the importance of individualism and collectivism in the assessment, diagnosis, and treatment of client and client systems (Sue, 2001).

This therapeutic perspective integrates the impact of cultural and social norms, starting at the beginning of treatment. Therapists who use this perspective work with clients to obtain and integrate information about their cultural patterns into a unique treatment approach based on their particular situation (Stewart, Simmons, & Habibpour, 2012). Sociocultural therapy can include individual, group, family, and couples treatment modalities.



Figure 4.2.1: How do your cultural and religious beliefs affect your attitude toward mental health treatment? (credit “top-left”: modification of work by Staffan Scherz; credit “top-left-middle”: modification of work by Alejandra Quintero Sinisterra; credit “top-right-middle”: modification of work by Pedro Ribeiro Simões; credit “top-right”: modification of work by Agustin Ruiz; credit “bottom-left”: modification of work by Czech Provincial Reconstruction Team; credit “bottom-left-middle”: modification of work by Arian Zwegers; credit “bottom-right-middle”: modification of work by “Wonderlane”/Flickr; credit “bottom-right”: modification of work by Shiraz Chanawala)

Barriers to Treatment

Statistically, ethnic minorities tend to utilize mental health services less frequently than White, middle-class Americans (Alegría et al., 2008; Richman, Kohn-Wood, & Williams, 2007). Why is this so? Perhaps the reason has to do with access and availability of mental health services. Ethnic minorities and individuals of low socioeconomic status (SES) report that barriers to services include lack of insurance, transportation, and time (Thomas & Snowden, 2002). However, researchers have found that even when income levels and insurance variables are taken into account, ethnic minorities are far less likely to seek out and utilize mental health services. And when access to mental health services is comparable across ethnic and racial groups, differences in service utilization remain (Richman et al., 2007).

In a study involving thousands of women, it was found that the prevalence rate of anorexia was similar across different races, but that bulimia nervosa was more prevalent among Hispanic and African American women when compared with non-Hispanic whites (Marques et al., 2011). Although they have similar or higher rates of eating disorders, Hispanic and African American women with these disorders tend to seek and engage in treatment far less than Caucasian women. These findings suggest ethnic disparities in access to care, as well as clinical and referral practices that may prevent Hispanic and African American women from receiving care, which could include lack of bilingual treatment, stigma, fear of not being understood, family privacy, and lack of education about eating disorders.

Perceptions and attitudes toward mental health services may also contribute to this imbalance. A recent study at King's College, London, found many complex reasons why people do not seek treatment: self-sufficiency and not seeing the need for help, not seeing therapy as effective, concerns about confidentiality, and the many effects of stigma and shame (Clement et al., 2014). And in another study, African Americans exhibiting depression were less willing to seek treatment due to fear of possible psychiatric hospitalization as well as fear of the treatment itself (Sussman, Robins, & Earls, 1987). Instead of mental health treatment, many African Americans prefer to be self-reliant or use spiritual practices (Snowden, 2001; Belgrave & Allison, 2010). For example, it has been found that the Black church plays a significant role as an alternative to mental health services by providing prevention and treatment-type programs designed to enhance the psychological and physical well-being of its members (Blank, Mahmood, Fox, & Guterbock, 2002).

Additionally, people belonging to ethnic groups that already report concerns about prejudice and discrimination are less likely to seek services for a mental illness because they view it as an additional stigma (Gary, 2005; Townes, Cunningham, & Chavez-Korell, 2009; Scott, McCoy, Munson, Snowden, & McMillen, 2011). For example, in one recent study of 462 older Korean Americans (over the age of 60) many participants reported suffering from depressive symptoms. However, 71% indicated they thought depression was a sign of personal weakness, and 14% reported that having a mentally ill family member would bring shame to the family (Jang, Chiriboga, & Okazaki, 2009).

Language differences are a further barrier to treatment. In the previous study on Korean Americans' attitudes toward mental health services, it was found that there were no Korean-speaking mental health professionals where the study was conducted (Orlando and Tampa, Florida) (Jang et al., 2009). Because of the growing number of people from ethnically diverse backgrounds, there is a need for therapists and psychologists to develop knowledge and skills to become culturally competent (Ahmed, Wilson, Henriksen, & Jones, 2011). Those providing therapy must approach the process from the context of the unique culture of each client (Sue & Sue, 2007).

DIG DEEPER: Treatment Perceptions

By the time a child is a senior in high school, 20% of his classmates—that is 1 in 5 —will have experienced a mental health problem (U.S. Department of Health and Human Services, 1999), and 8% —about 1 in 12 —will have attempted suicide (Centers for Disease Control and Prevention, 2014). Of those classmates experiencing mental disorders, only 20% will receive professional help (U.S. Public Health Service, 2000). Why?

It seems that the public has a negative perception of children and teens with mental health disorders. According to researchers from Indiana University, the University of Virginia, and Columbia University, interviews with over 1,300 U.S. adults show that they believe children with depression are prone to violence and that if a child receives treatment for a psychological disorder, then that child is more likely to be rejected by peers at school.

Bernice Pescosolido, author of the study, asserts that this is a misconception. However, stigmatization of psychological disorders is one of the main reasons why young people do not get the help they need when they are having difficulties. Pescosolido and her colleagues caution that this stigma surrounding mental illness, based on misconceptions rather than facts, can be devastating to the emotional and social well-being of our nation's children.

This warning played out as a national tragedy in the 2012 shootings at Sandy Hook Elementary. In her blog, Suzy DeYoung (2013), co-founder of Sandy Hook Promise (the organization parents and concerned others set up in the wake of the school massacre) speaks to treatment perceptions and what happens when children do not receive the mental health treatment they desperately need.

I've become accustomed to the reaction when I tell people where I'm from.

Eleven months later, it's as consistent as it was back in January.

Just yesterday, inquiring as to the availability of a rental house this holiday season, the gentleman taking my information paused to ask, “Newtown, CT? Isn’t that where that...that thing happened?”

A recent encounter in the Massachusetts Berkshires, however, took me by surprise.

It was in a small, charming art gallery. The proprietor, a woman who looked to be in her 60s, asked where we were from. My response usually depends on my present mood and readiness for the inevitable dialogue. Sometimes it’s simply, Connecticut. This time, I replied, Newtown, CT.

The woman’s demeanor abruptly shifted from one of amiable graciousness to one of visible agitation.

“Oh my god,” she said wide eyed and open mouthed. “Did you know her?”

....

“Her?” I inquired

That woman,” she replied with disdain, “that woman that raised that monster.”

“That woman’s” name was Nancy Lanza. Her son, Adam, killed her with a rifle blast to the head before heading out to kill 20 children and six educators at Sandy Hook Elementary School in Newtown, CT last December 14th.

When Nelba Marquez Greene, whose beautiful 6-year-old daughter, Ana, was killed by Adam Lanza, was recently asked how she felt about “that woman,” this was her reply:

“She’s a victim herself. And it’s time in America that we start looking at mental illness with compassion, and helping people who need it.”

“This was a family that needed help, an individual that needed help and didn’t get it. And what better can come of this, of this time in America, than if we can get help to people who really need it?” (pars. 1–7, 10–15)

Fortunately, we are starting to see campaigns related to the destigmatization of mental illness and an increase in public education and awareness. Join the effort by encouraging and supporting those around you to seek help if they need it. To learn more, visit the National Alliance on Mental Illness (NAMI) website (<http://www.nami.org/>). The nation’s largest nonprofit mental health advocacy and support organization is NAMI.

Summary

The sociocultural perspective looks at you, your behaviors, and your symptoms in the context of your culture and background. Clinicians using this approach integrate cultural and religious beliefs into the therapeutic process. Research has shown that ethnic minorities are less likely to access mental health services than their White middle-class American counterparts. Barriers to treatment include lack of insurance, transportation, and time; cultural views that mental illness is a stigma; fears about treatment; and language barriers.

Glossary

cultural competence

therapist’s understanding and attention to issues of race, culture, and ethnicity in providing treatment

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